

QUALITY & SAFETY PHARMACY PROGRAM

User Assessment Guide

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a. The QCPP team
b. The Guild Clinical Governance Committee
c. The Impartiality Sub-committee.

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ACKNOWLEDGEMENT OF COUNTRY

The Pharmacy Guild of Australia acknowledges the Traditional Custodians of this nation's lands, waters, and skies. The Guild pays its respects to the Aboriginal and Torres Strait Islander people, their diverse languages, their cultural practices and recognises their continuing connection and contribution to the lands on which we work and live.

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Foreword

Welcome to the Quality & Safety Pharmacy Program (QSPP) User Assessment Guide.

For almost 30 years, the Quality Care Pharmacy Program (QCPP) has provided assurance that community pharmacies across Australia operate to consistent quality standards and deliver safe, reliable care. Since its introduction in 1997, the QCPP has played an important role in supporting quality improvement and strengthening public confidence in community pharmacy.

Community pharmacy practice continues to evolve. Pharmacies now play an increasingly important role within Australia's primary healthcare system, delivering a broader range of clinical services, supporting medicine safety, improving access to care, and helping consumers manage their health closer to home. As pharmacy scope of practice expands, so too does the need for robust systems that support safe, high-quality and patient-centred care.

The transition from QCPP to QSPP reflects this evolution. Aligned with the Australian Community Pharmacy Standard AS 85000:2024 and the National Safety and Quality Primary and Community Healthcare Standards, QSPP provides a contemporary accreditation framework that places greater emphasis on clinical governance, patient safety, consumer partnership, quality improvement and organisational accountability.

The transition to QSPP on 1 October 2026 marks an important milestone for Australian community pharmacy. Building on the strong foundations established by QCPP, QSPP has been designed to better reflect the evolving role of community pharmacy within the healthcare system and support the safe delivery of an expanding range of services. It provides a practical and proportionate framework that helps pharmacies embed quality and safety into everyday practice while continuing to meet the needs and expectations of patients, communities and healthcare partners.

Importantly, QSPP moves beyond a compliance-focused approach to support pharmacies in embedding quality, safety and continuous improvement into everyday practice. By strengthening governance, risk management, workforce capability and quality improvement processes, the program helps pharmacies consistently deliver safe and effective services while responding to the changing needs of their communities.

As pharmacist scope of practice continues to evolve, pharmacy-level assurance will become increasingly important in maintaining public confidence and demonstrating that services are delivered within a framework of quality, safety and accountability. QSPP provides that assurance by supporting pharmacies to meet nationally recognised standards and continuously improve the care they provide.

This User Assessment Guide has been designed as a practical resource to help pharmacies understand the intent of the Standard, implement its requirements and prepare for accreditation. It translates the requirements into practical actions, reflective questions, evidence examples and implementation guidance that can be applied in day-to-day practice.

This edition incorporates insights gained through sector consultation, implementation activities and stakeholder feedback following the release of Version 1 in March 2026, helping to further refine and strengthen the guidance provided to pharmacies as they prepare for the transition to QSPP.

Whether your pharmacy is continuing its quality improvement journey and preparing for the transition to QSPP or preparing for its first QSPP assessment, I encourage you to use this guide as a key reference for understanding the Standard, strengthening your systems and supporting safe, high-quality care for every patient. Through a shared commitment to excellence, clinical governance, continuous improvement and patient-centred care, we can continue to strengthen the safety, quality and trust that communities place in Australian community pharmacy.

Natalie Willis

Chair, Clinical Governance Committee

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Introduction

The Quality & Safety Pharmacy Program accreditation is a certification that recognises the minimum acceptable standard in community pharmacy practice as specified in AS 85000:2024 Australian Community Pharmacy Standard. Achieving accreditation against this standard demonstrates a commitment to excellence and quality in patient care.

Overview of AS 85000:2024 Australian Community Pharmacy Standard

What is AS 85000:2024 Australian Community Pharmacy Standard?

AS 85000:2024 Australian Community Pharmacy Standard is a comprehensive standard that outlines best practice requirements for community pharmacies. It covers various aspects such as clinical governance, patient safety, quality service delivery, staff competencies, consumer engagement and operational efficiency.

Benefits of Accreditation

Enhanced Community Trust: Accreditation enhances community trust and provides assurance to patients and consumers of the pharmacy's adherence to high quality standards.

Operational Excellence: Implementing the Australian Community Pharmacy Standard can lead to improved operational efficiencies and service delivery.

Professional Recognition: Accredited pharmacies are recognised for their commitment to quality and excellence.

Risk Mitigation: Accreditation ensures an accountability framework for owners to meet their legislative obligations.

Access to Funding: Demonstrates value to funders.

Steps to Achieve Accreditation

Step 1: Initial Assessment

Conduct a self-assessment of your current pharmacy practices and compare them with the AS 85000:2024 Australian Community Pharmacy Standard. Identify any gaps or areas that need improvement.

Step 2: Staff Training and Development

Ensure that all pharmacy staff are adequately trained and understand the requirements of AS 85000:2024 Australian Community Pharmacy Standard. Provide ongoing professional development opportunities to maintain high standards.

Step 3: Implement Best Practices

Adopt and implement the best practices outlined in AS 85000:2024 Australian Community Pharmacy Standard. This may include updating policies, procedures and protocols, improving patient and consumer consultation processes, and ensuring appropriate record-keeping and business systems.

Step 4: Internal Audits

Conduct regular internal audits to ensure that all pharmacy systems and processes comply with the AS 85000:2024 Australian Community Pharmacy Standard. These audits are crucial for identifying areas of improvement and maintaining the highest levels of safety and quality in pharmacy operations. Use the audit findings to implement continuous quality improvement measures, ensuring that the pharmacy's practices remain safe, efficient, and contemporary.

Step 5: External Audit and Accreditation

Once you are confident that your pharmacy meets the standards, apply for accreditation through the Quality & Safety Pharmacy Program. If successful, your pharmacy will receive the accreditation against AS 85000:2024 Australian Community Pharmacy Standard which demonstrates the community pharmacy's commitment to patient safety, quality service delivery, staff competencies, consumer engagement and operational efficiency.

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Maintaining Accreditation: Continuous improvement

Accreditation is not a one-time process but an ongoing commitment to excellence in pharmacy practice. Continuous improvement is essential for maintaining accreditation, which requires staying updated with evolving standards and undergoing regular re-accreditation. By prioritising continuous quality improvement, pharmacies can ensure better patient outcomes, enhanced safety protocols, and more effective care and efficient business practices.

Re-accreditation

This process underscores the pharmacy's dedication to excellence in healthcare and commitment to continuous quality improvement. Regular audits will play a crucial role in assessing and enhancing the quality of services provided, ensuring that the pharmacy consistently meets the highest standard of patient care and safety.

Conclusion

Achieving accreditation against AS 85000:2024 Australian Community Pharmacy Standard reflects dedication to patient safety, service quality, staff competencies, consumer engagement and operational efficiency. By following this guide, your pharmacy can successfully navigate the Quality & Safety Pharmacy Program accreditation process and reap the numerous benefits it offers.

Resources

AS 85000:2024 Australian Community Pharmacy Standard: Obtain a copy of the Standard from the Quality & Safety Pharmacy Program Knowledge Hub or purchase directly from Standards Australia.

Knowledge Hub: Providing accredited community pharmacies with a range of helpful tools and resources to support the accreditation process.

Professional Quality Consultation Services: Consider hiring a consultant specialising in clinical governance and establishing systems and processes to support pharmacy accreditation and ongoing continuous quality improvement.



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Quality & Safety Pharmacy Program User Assessment Guide

This document serves as a comprehensive guide for community pharmacies seeking Quality & Safety Pharmacy Program (QSPP) accreditation against the AS 85000:2024 Australian Community Pharmacy Standard. It outlines the standard, the benefits, and the necessary steps community pharmacies need to undertake to achieve and maintain accreditation. This guide provides context and understanding to support community pharmacies to understand and embed clinical governance, patient safety, quality service delivery, staff competence, consumer engagement and operational efficiency into community pharmacy practice and drive improved patient and consumer outcomes.

The guide is divided into four key domains:

Domain 1 – Clinical Governance and Leadership

Domain 2 – Safely Delivering Medications and Pharmacy Services

Domain 3 – Partnering with Consumers

Domain 4 – Business Governance and Operations

The four domains serve as a structured framework to ensure community pharmacies can comprehensively address all aspects of AS 85000:2024 within their business operations and service delivery.

Each domain targets a particular key area essential to achieving the overarching goals of the AS 85000:2024 Australian Community Pharmacy Standard. By organising the standard into these domains, the framework ensures that pharmacies can focus on specific aspects of governance, service quality, consumer engagement, and operational excellence.



Domain 1: Clinical Governance and Leadership - emphasises the importance of strong leadership and systems for maintaining patient safety and clinical standards.



Domain 2: Safely Delivering Medications and Pharmacy Services - focuses on ensuring safe and effective dispensing practices to support the delivery of quality pharmaceutical care and services.



Domain 3: Partnering with Consumers and the Community - underlines the significance of community engagement and collaboration to foster trust and deliver personalised care.



Domain 4: Business Governance and Operations - addresses the efficient and sustainable running of pharmacy businesses, ensuring their long-term viability, compliance and effective risk management.

Together, these domains provide a holistic approach that aligns patient needs, professional requirements, and operational challenges, driving improved outcomes for both patients /consumers and the pharmacy team.

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Mapping AS 85000:2024 Australian Community Pharmacy Standard to the QSPP domains

Domain 1

Clinical Governance and Leadership

- 1.1 Governance, leadership, and culture (1.1.1)*
- 1.2 Patient safety and quality systems (1.1.2)*
- 1.3 Clinical performance and effectiveness (1.1.3)*
- 1.4 Safe environment for the delivery of care (1.1.4)*

Domain 2

Safely Delivering Medications and Pharmacy Services

- 2.1 Delivery of pharmacy services (2.1.1)*
- 2.2 Prescribing by pharmacists (if offered) (2.1.2)*
- 2.3 Safe and appropriate provision of medicines, medical devices and poisons (2.2.2)*
- 2.4 Complex compounding (if offered) (2.2.3)*
- 2.5 Indirect provision of medicines (2.2.4)*
- 2.6 Preventing and controlling infections (2.3.1)*
- 2.7 Communicating for safety (2.3.3)*
- 2.8 Recognising and responding to serious deterioration and minimising harm (2.3.4)*

Domain 3

Partnering with Consumers

- 3.1 Partnering with patients in their own care (1.1.5)*
- 3.2 Health literacy (1.1.6)*
- 3.3 Partnering with consumers in pharmacy design and governance (1.1.7)*
- 3.4 Comprehensive care (2.3.2)*

Domain 4

Business Governance and Operations

- 4.1 Legal and professional obligations (1.2.1)*
- 4.2 Business viability (1.2.2)*
- 4.3 Business continuity plan (1.2.3)*
- 4.4 Pharmacy premises (1.2.4)*
- 4.5 Safety and security (1.2.5 & 2.4.2)*
- 4.6 Information technology (IT) (1.2.6 & 2.4.3)*
- 4.7 Advertising of pharmacy services and therapeutic goods (2.1.3 & 2.2.5)*
- 4.8 Stock management (2.2.1)*
- 4.9 Human resources (2.4.1)*

** numbers in brackets refers to the AS 85000:2024 sub-clause*

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Domain Structure

The domain structure provides a systematic framework to assist community pharmacies to implement the standard effectively, and includes clear components of:

Intention

The overarching goal or aim of the standard subclause for the community pharmacy.

Outcome

The result or impact from a patient and consumer perspective when the standard subclause is implemented effectively.

Standard Subclause

A concise description of the actions required to meet a specific subclause directly from AS 85000:2024 Australian Community Pharmacy Standard.

Key Tasks

Lists the critical activities that need to be performed to comply with the standard subclause. These tasks link directly back to *AS 85000:2024*.

Reflective Questions

Open ended questions designed to encourage self-assessment and reflection to evaluate current practices and the pharmacy alignment with standard subclauses.

Monitoring and Evidence Examples

Specific examples or documentation that show how a standard subclause is met. The type and range of evidence provided will vary depending on each individual community pharmacy's size, structure, the services it offers and the community it serves.

Resources and Information

Helpful materials and information that support the understanding and application of a standard subclause.

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Domain 1.

Clinical Governance and Leadership



Pharmacies play a vital role in the healthcare system, serving as accessible destinations for medication management, health advice, and patient care. As the scope of pharmacy practice continues to expand, so too does the need for robust clinical governance that ensures safety, accountability, and continuous improvement. This domain provides a structured framework to support pharmacy professionals in delivering high-quality care that is both evidence-based and patient-centred.

The Clinical Governance and Leadership Domain consists of four key sections:

1.1 **Governance, Leadership, and Culture**

1.2 **Patient Safety and Quality Systems**

1.3 **Clinical Performance and Effectiveness**

1.4 **Safe Environment for the Delivery of Care**

Together, these components form a cohesive governance framework that supports excellence in pharmacy practice. By adopting these principles, community pharmacies commit to a culture of safety, accountability, and continuous improvement—ultimately enhancing outcomes for the communities they serve.

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1.1 Governance, Leadership, and Culture

Intention

The safety and quality of care provided by the pharmacy is continuously improved through robust clinical governance systems and collaboration with patients and other healthcare providers.

Outcome

The pharmacy services I receive are high quality and continuously improving.

Standard Subclause

1.1.1.1 The pharmacy shall:

a. Have a culture of safety and quality improvement.

b. Partner with patients, carers, consumers and communities.

c. Establish and maintain systems for integrating care with other service providers involved in a patient's care.

d. Set priorities for safe and quality clinical care and ensure that these are communicated effectively to pharmacy staff.

e. Establish and maintain a clinical governance framework.

f. Clearly define the safety and quality roles, responsibilities and accountabilities of those who have ownership of the pharmacy, management, and pharmacy staff.

g. Monitor and review the safety and quality performance of the pharmacy.

h. Consider the safety and quality of health care for patients in its business decision-making.

Reflective Questions

- How do clinical governance policies align with the latest safety and quality legislation and regulations?
- How are we fostering a culture of safety and quality in our pharmacy?
- What safety and quality concerns are present in our pharmacy?
- Are there areas of safety and quality performance that we should prioritise as a pharmacy team?
- What measures have we taken to support our pharmacy staff in understanding and fulfilling their roles in maintaining safe and quality performance?
- How do we partner with patients, carers, consumers, and communities to integrate their feedback into our safety and continuous quality improvement processes?
- What performance metrics do we use to monitor and review our safety and quality performance?
- How can we facilitate seamless coordination of care with other healthcare providers and organisations?
- When making pharmacy business decisions, do we consider the safety and quality of the healthcare we are providing?

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Key Tasks

- Clearly document and communicate the safety and quality roles, responsibilities and accountabilities of pharmacy owners, managers and other staff members to embed a culture of safety and continuous quality improvement in the pharmacy.
- Implement systems in the pharmacy to encourage open communication and continued analysis and learning amongst staff, in order to prioritise patient safety as a core value.
- Use information from staff, patients, consumers and other healthcare providers to identify safety and quality risks and priorities, and address and monitor these as a team.
- Implement robust monitoring and review mechanisms to regularly assess the pharmacy's performance against safety and quality metrics, and to identify areas for improvement.
- Make business decisions that prioritise the safety and quality of pharmacy services for patients.

Monitoring and Evidence Examples

- Annual Attestation Statement confirming compliance and commitment with the Quality & Safety Pharmacy Program.
- Documented evidence, including position descriptions and / or employment contracts, that clearly outline the accountabilities and responsibilities of pharmacy owner(s), managers and pharmacy staff in the delivery of safe, quality care.
- Demonstrated evidence of communications channels (e.g. staff communication books, messaging platforms, pin-board notices) and how they are used to improve safety, quality and continuous learning among staff.
- Evidence the pharmacy has considered safety and quality priorities and concerns, including strategies the pharmacy has undertaken to address identified issues.
- The pharmacy's performance metrics are used to measure patient safety, service quality, and staff performance.
- Documented feedback from patients, consumers and communities related to the safety and quality of services provided. Evidence of improvements made based on the feedback provided.
- Evidence of activities demonstrating engagement with consumers to drive quality and safety.
- Evidence of strong communication channels and relationships with other local healthcare providers and community organisations to ensure coordinated patient care.
- Minutes of meetings, business plans, or emails that reflect business decisions where patient safety and quality have been considered and strategies implemented to improve outcomes.
- Documented evidence of audits of relevant laws, regulations, and professional standards related to safety and quality.
- The pharmacy demonstrates use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures.

Resources and Information

Australian Commission on Safety and Quality in Healthcare

- National Safety and Quality Primary and Community Healthcare Standards
- National Model Clinical Governance Framework

Pharmaceutical Society of Australia

- Clinical Governance Principles



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1.2 Patient Safety and Quality Systems

Intention

Integrating safety and quality systems into the pharmacy's governance framework allows for effective management and staff engagement in improving patient care.

Outcome

I know the care that I receive is safe and appropriate.

Standard Subclause

1.1.2.1 To mitigate risk, the pharmacy shall:

a. Establish and maintain policies, procedures and protocols and incorporate these into onboarding of pharmacy staff.

b. Make policies, procedures and protocols easily available to pharmacy staff.

c. Monitor and take action to improve adherence to policies, procedures and protocols.

d. Review all policies, procedures and protocols at least annually to ensure they remain fit for purpose.

e. Communicate changes of policies, procedures, and protocols to pharmacy staff.

f. Ensure compliance with relevant safety and quality legislation, regulations and jurisdictional requirements, as well as professional standards, guidelines and codes of practice.

1.1.2.2 The pharmacy shall use a range of information, including from pharmacy staff, patients, consumers and other healthcare providers, to:

a. Identify priorities for safety and quality improvement.

b. Implement and monitor safety and quality improvement activities.

c. Measure changes in safety and quality outcomes.

d. Provide timely information on safety and quality performance to patients, consumers and pharmacy staff.

Note:

Safety and quality improvements in pharmacy may encompass clinical and non-clinical activities. Information and its interpretation should be reviewed in collaboration with patients and communities to promote shared decision-making and involvement in care.

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1.1.2.3 To manage risks, the pharmacy shall:

a. Support pharmacy staff to identify, mitigate and manage safety and quality risks.

b. Document and routinely monitor safety and quality risks.

c. Plan for, and manage, ongoing service provision during internal and external emergencies and disasters.

1.1.2.4 The pharmacy shall have an incident management system that:

a. Supports pharmacy staff to recognise and report incidents and near misses that occur inside, or outside, the pharmacy.

b. Supports patients, carers, families and other health professionals to communicate concerns or report incidents.

c. Involves pharmacy staff in the review of incidents.

d. Provides timely feedback on the analysis of incidents to pharmacy staff, patients and consumers who have communicated concerns or incidents.

e. Uses the information from the analysis of incidents to improve safety and quality.

f. Incorporates risks identified in the analysis of incidents into the risk management system.

g. Regularly reviews and acts to improve the effectiveness of the incident management and investigation systems.

h. Reports incidents to third parties (for example, indemnity insurers, regulatory bodies, pharmaceutical companies etc.) as appropriate.

Note:

The incident management system should focus on improving safety at a system level and should be underpinned by a no-blame approach, allowing pharmacy staff to speak openly about concerns and mistakes.



Standard Subclause

1.1.2.5 The pharmacy shall use the current Australian Open Disclosure Framework when a patient is harmed through the delivery of health care.

1.1.2.6 The pharmacy shall:

a. Seek feedback from patients and consumers about their experiences and outcomes of health care.

b. Have processes to regularly seek feedback from pharmacy staff on their understanding and use of the safety and quality system.

c. Use feedback to improve safety and quality.

1.1.2.7 The pharmacy shall:

a. Provide opportunities for its patients to report negative feedback.

b. Have processes to address negative feedback in a timely way.

c. Use information from the analysis of negative feedback to improve safety and quality.

Note:

Negative feedback may be received in many ways, including a formal complaint, informal discussion, on social media or in online reviews. The pharmacy should implement a mechanism to flag incidents and feedback as clinical or non-clinical to assist with analysis.

1.1.2.8 The pharmacy shall identify patient populations using its service with distinct needs, priorities and preferences, including:

a. People of Aboriginal and/or Torres Strait Islander origin.

b. People with a disability.

c. People with diverse backgrounds.

d. Other populations and communities identified.

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Standard Subclause

1.1.2.9 The pharmacy shall use information on its patient populations, and collaborate with them, to inform planning and delivery of care that is tailored to their needs and preferences:

a. Information gathered from patient populations shall be obtained with appropriate consent.

1.1.2.10 The pharmacy shall have a healthcare record system that:

a. Makes patient medication and health records available to pharmacists at the point of dispensing and service delivery.

b. Supports pharmacy staff to maintain accurate and complete records.

c. Complies with privacy and security regulations for the handling, storage and disposal of records.

d. Supports audits of records.

e. Facilitates a patient's access to their healthcare records.

Note:

Vigilance is required when handling, storing and disposing of all types of records in the pharmacy including, but not limited to, patient healthcare records, employment records and financial records.

1.1.2.11 The pharmacy shall have processes to:

a. Receive and review reports and records on patients (for example, Home Medicines Review reports, hospital discharge summaries, medication charts, pathology reports, real-time prescription monitoring information).

b. Recall patients and communicate about reports, records and healthcare options.

c. Take action and/or respond to reports and records in a timely manner.

d. Facilitate interprofessional collaboration through sharing reports and records as appropriate.

e. Document reports and records in a patient's healthcare record where possible.

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Standard Subclause

1.1.2.12 Pharmacies using a national electronic health record system (for example, My Health Record) shall have processes to:

a. Use national healthcare identifiers for patients and healthcare providers.

b. Support pharmacists to use the national electronic health record system to optimise the safety and quality of health care for patients.

1.1.2.13 Pharmacies providing clinical information to a national electronic health record system shall have processes to:

a. Comply with legislative requirements, standards, codes and guidelines.

b. Ensure the accuracy and completeness of information uploaded.

Reflective Questions

- How do we maintain and regularly update policies and procedures addressing safety and quality?
- How do we actively identify, monitor and mitigate risks to patients, staff and the pharmacy business?
- What strategies do we have in place that encourages transparent incident management reporting and timely feedback to staff on incidents and near misses?
- How could we demonstrate the improvements the pharmacy has implemented based on incident data being documented, analysed and evaluated?
- How do we adopt principles from the Australian Open Disclosure Framework when a patient safety incident occurs in our pharmacy or as a result of our care?
- How do we ensure the patient healthcare records we keep are comprehensive, accurate, and easily accessible to pharmacists when providing care?
- What mechanisms are in place to receive patient reports or records electronically?
- How do we ensure compliance with legislative requirements when providing clinical information to national electronic health record systems?
- Do we actively gather and incorporate feedback from patients, consumers, and staff to enhance safety and quality of care?
- What policies, procedures, and protocols do our pharmacy staff have access to? How is it ensured that all staff understand and act in accordance with these policies, procedures and protocols?
- What systems do we have in place to monitor and improve adherence to our policies, procedures, and protocols?
- How effectively do we communicate changes in policies, procedures, and protocols to our pharmacy staff?
- What mechanisms do we use to identify specific patient populations who visit the pharmacy? Are there barriers or challenges for these patient populations in accessing the pharmacy's services? How can we reduce barriers to accessing healthcare for these patients?
- What strategies do we have in place to tailor the care we provide to meet the distinct needs of patient populations who attend the pharmacy?

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Key Tasks

- Maintain comprehensive, regularly reviewed and updated safety and quality policies, procedures, and protocols that are easily accessible to staff, with clear communication of any changes.
- Provide regular staff training on key safety and quality policies, procedures and protocols to encourage adherence and understanding of clinical governance.
- Conduct routine audits of relevant safety and quality legislation, professional standards, guidelines and codes of practice, to ensure the pharmacy's policies, procedures and protocols remain compliant.
- Identify, document, and monitor safety and quality risks consistently and continuously.
- Provide resources and training to pharmacy staff on risk identification, risk management and risk mitigation strategies.
- Establish, maintain and follow a robust, documented system for detecting, assessing, responding and reporting incidents and near misses within the pharmacy. Encourage transparent communication (as per the Australian Open Disclosure Framework) and provide timely feedback on incident analysis, including any improvements implemented based on the incident data analysis.
- Adopt a no-blame approach to foster a culture of openness and proactive problem-solving in the pharmacy.
- Train staff on the pharmacy's incident management system and open disclosure principles.
- Implement systems to proactively gather feedback from patients, consumers and staff (e.g. feedback forms, surveys, conversations, follow-up messages or phone calls). Feedback received (both positive and negative) should be documented and responded to in a timely manner, including the identification and implementation of improvements in the pharmacy's processes, as appropriate.
- Identify distinct needs of specific patient populations, such as Aboriginal and/or Torres Strait Islander individuals, culturally diverse groups or people with disabilities (appropriate to the pharmacy community), and collaborate to tailor care based on their preferences and priorities.
- Provide resources and training to pharmacy staff to improve awareness and understanding of the different patient populations who access the pharmacy and how best to meet their needs and preferences.
- Establish, implement and maintain a healthcare record management system to record comprehensive and accurate medication and patient information. Dispensing and other patient records need to be:
  - **Kept in compliance with the Australian Privacy Principles and data security regulations**
  - **Accessible to pharmacists at the point of care**
  - **Audited regularly for completeness and accuracy**
  - **Provided to patients if required or requested.**
- Implement a process to receive, review, respond, document and share (with appropriate consent) patient reports or records (e.g. Home Medicine Review reports, hospital discharge summaries, medication charts, telehealth referrals, consultation records).
- Develop and embed a protocol for utilising a national electronic health record system (e.g. My Health Record) that encourages the use of national healthcare identifiers for patients and healthcare providers, standardised clinical terminology, is compliant with legislation and data security guidelines, and supports safe and quality care for patients.
- Provide training and education to staff members accessing a national electronic health record system to access and upload information accurately and consistently.

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Monitoring and Evidence Examples

- Records of compliance audits, reviews and updates to the pharmacy's policies, procedures and protocols.
- Evidence of staff training and education related to the pharmacy's:
  - Policies, procedures and protocols
  - Risk management system
  - Incident management system
  - Open disclosure principles
  - Patient populations and their distinct needs and preferences
  - Documentation and the use of a national electronic health records system
- A risk management plan identifying and mitigating safety and quality risks.
- Evidence of the pharmacy's incident management system including detection of incidents and near misses, actions taken, outcomes, appropriate reporting, and improvements implemented following the analysis of incidents and near misses.
- Demonstrated evidence, and examples, of how the principles from the Australian Open Disclosure Framework have been embedded into the pharmacy's incident response procedures.
- Records of meetings or communications with pharmacy staff where incidents have been discussed and strategies and actions communicated to reduce the likelihood of incidents recurring.
- Demonstrated evidence of how the pharmacy seeks feedback from patients, consumers and staff, this may include records of feedback received, responses provided, and improvements made (e.g. patient reported questionnaires, survey results, completed feedback forms).
- ABS census data for the pharmacy's local area and evidence of the specific populations (e.g. Aboriginal and/or Torres Strait Islander People, LGBTQIA+ people, elderly people, people with a disability) that access the pharmacy.
- Evidence (e.g. cultural awareness policy, tailored care plans) of how the pharmacy tailors the care it provides to the needs and preferences of local community and patient populations.
- Demonstrated evidence that resources are available and accessible to patients from diverse backgrounds (e.g. health information in different formats and languages).
- Demonstrated evidence of the pharmacy's healthcare record management system and processes to ensure accurate healthcare records.
- Privacy and Data Collection statements are published and easily accessible for staff, patients and consumers.
- Demonstrated evidence of how the pharmacy receives, reviews, responds, documents and shares patient reports and records received by the pharmacy.
- The pharmacy's protocol for utilising a national electronic health record system.
- Registration and certification records verifying the pharmacy's authority to contribute to a national electronic health record system.
- The pharmacy demonstrates use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures.
- Implement and operate an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures.

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Resources and Information

Australian Commission on Safety and Quality in Healthcare

- Incident Management Guide
- The Australian Open Disclosure Framework
- Australian Charter of Healthcare Rights

Pharmaceutical Society of Australia

- Guidelines for pharmacists supporting Aboriginal and Torres Strait Islander peoples with Medicines Management

Australian Government Office of the Australian Information Commissioner

- Australian Privacy Principles

Australian Digital Health Agency

- My Health Record

Australian Indigenous Health Info Net

- Cultural Safety for Health Professionals - Pharmacist



1.3 Clinical Performance and Effectiveness

Intention

Community pharmacies deliver safe, high-quality healthcare through robust clinical processes, continuous performance reviews, and adherence to legislative and cultural safety standards.

Outcome

I receive safe and high-quality effective care that is delivered by trained professional staff.

Standard Subclause

Pharmacy staff shall have the right qualifications, knowledge and skills to provide safe, quality health care to patients.

1.1.3.1 The pharmacy shall:

a. Have processes to support pharmacy staff to understand and fulfil their assigned safety and quality roles and responsibilities.

b. Provide pharmacy staff with induction and training to their safety and quality roles on commencement with the pharmacy (i.e., onboarding), when safety and quality responsibilities change and when new pharmacy services are introduced.

c. Identify the training needs of pharmacy staff to meet the requirements of this standard.

d. Ensure pharmacy staff complete training to meet their safety and quality training needs.

1.1.3.2 The pharmacy shall support pharmacy staff through training and resources to provide culturally safe services to meet the needs of its Aboriginal and/ or Torres Strait Islander patients and other culturally and linguistically diverse groups.

1.1.3.3 The pharmacy shall have valid and reliable review processes for pharmacy staff that:

a. Are used to regularly review their performance.

b. Identify needs for training and development of safety and quality.

1.1.3.4 The pharmacy shall have processes to ensure that pharmacists have the qualifications, knowledge and skills required to perform their role by:

a. Describing the scope of practice for pharmacists practising in the pharmacy.

b. Monitoring practices to ensure pharmacists are operating within their scope of practice.

c. Reviewing pharmacists' scope of practice when a pharmacy service, procedure or technology is introduced or substantially altered.

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Standard Subclause

1.1.3.5 To support evidence-based care, the pharmacy shall:

a. Provide its pharmacists with ready access to practice guidelines and available evidence, and decision support tools relevant to their clinical practice.

b. Require its pharmacists to use practice guidelines, available evidence and decision support tools relevant to their clinical practice.

1.1.3.6 The pharmacy shall support its pharmacists to:

a. Monitor and review care delivered against relevant evidence.

b. Explore reasons for variation of health care from relevant evidence.

c. Use information on unwarranted variation from evidence-based care to improve health care.

Reflective Questions

- What training programs are provided to pharmacy staff to address their safety and quality roles and responsibilities?
- What processes are in place to identify specific training needs for staff members, especially in areas such as cultural safety?
- What strategies are in place to ensure pharmacists consistently operate within their defined scope of practice?
- How are pharmacists supported to access evidence-based guidelines and decision support tools relevant to their clinical practices?
- How do we encourage pharmacists to use evidence-based resources during clinical decision-making?
- What measures does the pharmacy take to deliver culturally safe services to diverse groups, including Aboriginal and/or Torres Strait Islander patients?
- How can care provided by the pharmacy be monitored and evaluated against evidence-based guidelines?

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Key Tasks

- Deliver initial and ongoing safety and quality training for staff to ensure they are aware of their role responsibilities and compliance obligations, especially when roles change or new pharmacy services are introduced.
- Promote cultural safety through offering staff ongoing access to resources and training on delivering culturally safe services, particularly to Aboriginal and/or Torres Strait Islander patients and other diverse cultural groups, appropriate to the community.
- Define and document the scope of practice for pharmacists working in the pharmacy, noting this may vary between pharmacists. Monitor practices to ensure pharmacists operate within their defined scope, and review pharmacists' scope of practice if a new service or technology is introduced.
- Implement a feedback mechanism for pharmacists, or other healthcare providers, to report challenges or uncertainties related to delivering pharmacy services and/or scope of practice.
- Provide pharmacists with easy access to current, evidence-based clinical resources and support pharmacists to use these resources as part of their clinical decision-making.
- Regularly audit how pharmacy services are delivered and if it aligns with relevant evidence. Where there are deviations, assess whether the variations are warranted (e.g. adapted to meet a certain population need) or unwarranted (e.g. patients are not getting appropriate care) and utilise these insights to improve ongoing pharmacy practices.

Monitoring and Evidence Examples

- Records or evidence of ongoing training for pharmacy staff to ensure safe, quality, and culturally appropriate care.
- Evidence of collaboration with local community groups and/or Indigenous organisations to co-develop culturally appropriate service protocols or materials.
- Displays of visible signage or messaging in the pharmacy that acknowledges and respects the cultural diversity of the local community.
- Analysis of patient surveys that have been undertaken to gather feedback on cultural safety, with insights used for service improvement.
- Clearly documented policies and protocols outlining how pharmacists' scope of practice is managed and monitored.
- Documented evidence for scope of practice for pharmacists working in the pharmacy (e.g. position descriptions, contracts of employment or stand-alone documents).
- Records of feedback (e.g. patient reported measures, surveys, records of verbal feedback) related to delivering pharmacy services and/or scope of practice. For patient reported measures the pharmacy demonstrates use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures.
- Demonstrated evidence of how pharmacists' scope of practice is monitored and reviewed.
- Evidence of a centralised digital repository of current, evidence-based guidelines, accessible to all staff.
- Documented outcomes of clinical practice audits where service delivery was compared with best practice.

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• Resources and Information

Pharmaceutical Society of Australia

- Guidelines for pharmacists supporting Aboriginal and Torres Strait Islander peoples with Medicines Management

Australian Health Practitioner Regulation Agency (AHPRA)

- National Scheme's Aboriginal and Torres Strait Islander Health and Cultural Safety Strategy

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1.4 Safe Environment for the Delivery of Care

Intention

Community pharmacies provide safe, culturally sensitive, and high-quality healthcare in a well-maintained and equipped environment.

Outcome

I receive care in an environment that is clean, well-maintained, and equipped to meet my healthcare needs. My privacy is respected, and the cultural sensitivity of the staff ensures that I feel understood and valued.

Standard Subclause

The environment in which pharmacy services are delivered shall enable safe and quality health care for patients.

1.1.4.1 The pharmacy shall provide safe and appropriate care:

a. Through the design of the environment and management of the location where health care is provided.

b. By appropriately managing, and reviewing, the workload of pharmacy staff.

c. By providing access to an environment, devices and equipment that are fit for purpose and well maintained.

d. By safeguarding patients' privacy when health care is provided.

e. By respecting the cultural needs of patients when health care is provided.

f. By recognising and acknowledging trauma and its prevalence, and being sensitive to its dynamics when health care is provided.

g. By taking appropriate action in response to feedback regarding the environment in which health care was provided.

1.1.4.2 The pharmacy shall identify circumstances that have a high risk of potentially causing consumers to behave unpredictably and develop strategies to minimise the risks of harm to patients, consumers and pharmacy staff.

1.1.4.3 The pharmacy shall support all patients to access health care, including patients from diverse backgrounds, patients with a disability and other populations and communities identified.

1.1.4.4 The pharmacy shall provide a culturally safe environment that recognises the importance of the cultural beliefs and practices of Aboriginal and/or Torres Strait Islander people.

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Reflective Questions

- How does the design of the pharmacy environment support safe and appropriate health care delivery?
- In what ways are our devices and equipment maintained to ensure they remain fit for purpose?
- How are staff workloads managed to prevent burnout and ensure quality care for patients?
- What measures are in place to protect patient privacy within our pharmacy?
- How do we incorporate cultural sensitivity into the care we provide, particularly for Aboriginal and/or Torres Strait Islander patients?
- What strategies have we developed to address situations with high-risk behaviours from consumers?
- How accessible is our pharmacy for patients with disabilities or those from diverse cultural backgrounds?
- In what ways do we acknowledge and respond to feedback about our environment and services?

Key Tasks

- Regularly review the layout and design of the pharmacy to make sure it supports safe, accessible and appropriate care.
- Implement strategies to manage staff workload effectively, preventing burnout and ensuring the delivery of high-quality care and team support.
- Provide staff with access to programs aimed at supporting staff wellbeing (e.g. Employee Assistance Program, stress management resources and/or mental health support).
- Develop patient privacy safeguard measures so that areas where advice and services are delivered:
  - **Allow for private, confidential conversations with patient and consumers**
  - **Are respectful to cultural preferences**
  - **Are sensitive to patients who may have experienced previous trauma when receiving healthcare**
- Consider and identify the impact of trauma and high-risk situations that could lead to unpredictable patient behaviour. Develop protocols and train staff in strategies to deescalate situations and minimise risks to patients, staff, and the broader community.
- Create feedback systems to collect and respond to patient feedback, which could include feedback on the pharmacy environment and service delivery. For service delivery include the Implementation and operation of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures.
- Implement strategies to make the pharmacy accessible to all patients, including those living with disabilities and/or those from culturally and linguistically diverse backgrounds. This may involve implementing physical modifications, offering translated communications and culturally specific resources appropriate to the needs of the community.
- Actively foster a culturally safe environment by respecting and integrating the cultural beliefs and practices of Aboriginal and/or Torres Strait Islander people, and other prevalent community groups, into the pharmacy's design and practices.
- Facilitate access to diversity and inclusion training for pharmacy staff.

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Monitoring and Evidence Examples

- Clear signage and intuitive layouts are in place to guide patients and consumers efficiently through the pharmacy.
- Evidence of appropriately furnished and equipped consultation room/s to deliver pharmacy services and ensure confidential discussion.
- Demonstrated evidence of how the workload of pharmacy staff is monitored and managed.
- Evidence of wellbeing resources available to pharmacy staff.
- Evidence of a designated area(s) within the pharmacy to support patient interactions occurring in a private space which can accommodate individual needs of patients and consumers.
- Records or evidence of staff training and education related to trauma-informed care and high-risk situations, including protocols and strategies to manage unpredictable patient behaviour and de-escalate situations.
- Patient feedback related to the pharmacy environment and opportunities for improvement.
- Demonstrated evidence that entrances, aisles, and counters accommodate wheelchairs and mobility aids.
- Demonstrated evidence that pharmacy staff can offer bilingual or multilingual communication for diverse patient groups. Alternatively facilitate access to interpreter services or alternative communication methods for non-native speakers.
- Culturally inclusive materials are displayed within the pharmacy. Examples and evidence of non-verbal demonstrations of inclusion, for example display or imagery that reflects diversity in the community.
- Records of completed training by pharmacy staff related to diversity and inclusion.
- Evidence of participation in events of cultural significance to the local community.

Resources and Information

Australian Health Practitioner Regulation Agency (AHPRA)

- National Scheme's Aboriginal and Torres Strait Islander Health and Cultural Safety Strategy

Network of Alcohol and other Drugs Agencies (NADA)

- Trauma-informed practices for responding to difficult situations

Pharmaceutical Society of Australia

- Guidelines for pharmacists supporting Aboriginal and Torres Strait Islander peoples with Medicines Management

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Domain 2.

Safely Delivering Medications and Pharmacy Services



The AS 85000:2024 Australian Community Pharmacy Standard establishes a critical framework for ensuring the safe provision of medicines and pharmacy services while minimising clinical safety risks such as the spread of infection and fragmented care. By defining safety and quality parameters, the Standard supports a consistent and compliant approach to medication and service delivery that minimises risks, promotes positive health outcomes and fosters trust.

The key components in the Safely Delivering Medications and Pharmacy Services Domain are:

2.1 **Delivery of Pharmacy Services**

2.2 **Prescribing by Pharmacists**

2.3 **Safe and Appropriate Provision of Medicines, Medical Devices and Poisons**

2.4 **Complex Compounding**

2.5 **Indirect Provision of Medicines**

2.6 **Preventing and Controlling Infections**

2.7 **Communicating for Safety**

2.8 **Recognising and Responding to Serious Deterioration and Minimising Harm**

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As the breadth and complexity of pharmacy services expands in Australia, so does the need for increasingly robust systems for the delivery of care to ensure patient safety. As such, community pharmacies will receive tailored accreditation assessments under the Quality & Safety Pharmacy Program, reflective of the services they provide to their communities. For the purposes of assessment under QSPP, pharmacy services will be grouped into categories based on complexity and risks to patient safety, as follows:

General Pharmacy Services

General Pharmacy Services are well established and recognised in community pharmacies across Australia. To deliver these services safely, pharmacies are required to:

- Have access to current and appropriate clinical references, resources and equipment
- Deliver the services in a location and manner that protects patient privacy and confidentiality
- Implement policies, procedures and protocols to ensure consistent service delivery
- Keep detailed records of the care provided
- Review incidents, feedback and procedures regularly to improve the safety and quality of each service

Pharmacists delivering General Pharmacy Services operate within the foundational competency enabled by a pharmacy degree, however comprehensive medication management reviews and supply by protocol services require additional pharmacist training / accreditation.

Examples of General Pharmacy Services include:

- Dose Administration Aid (DAA) services
- Staged supply of medicines
- Opioid dependence treatment
- MedsCheck/ Diabetes MedsCheck
- Issuing absence from work certificates
- Home Medicines Reviews and Residential Medication Management Reviews
- Supplying medicines by protocol for conditions such as:
    - Urinary tract infections
    - Minor skin conditions including impetigo, psoriasis, shingles, acne and dermatitis
    - Musculoskeletal pain
    - Hormonal contraception resupply

Vaccination and Medicine Administration Services

Pharmacy services provided under this category involve the administration of therapeutic agents by injection following a detailed consultation. For safe service delivery, pharmacies must have the following:

- Pharmacist(s) with immunisation qualification, first aid, CPR and anaphylaxis certification
- Access to current and appropriate clinical references and resources
- Fit-for-purpose and well-maintained equipment including a purpose-built vaccine refrigerator, anaphylaxis response kit and sharps and clinical waste disposal units
- A consultation area providing visual and auditory privacy and confidentiality
- Policies, procedures and protocols to ensure consistent service delivery
- A system to facilitate detailed records of the care provided, and reporting to third parties as required
- Robust cold chain management procedures that align with the National Vaccine Storage Guidelines ‘Strive for 5’
- Infection control protocols
- Referral pathways to other healthcare providers and services
- Service evaluation processes to review incidents, feedback including patient reported experience measures (PREMs) and patient reported outcomes measures (PROMs) and jurisdictional requirements to improve the safety and quality of vaccination and medicine administration services

Examples of Vaccination and Medicine Administration Services include:

- Immunisation against influenza, Covid-19 and other conditions as per the National Immunisation Schedule
- Administration of denosumab
- Administration of long-acting buprenorphine
- Administration of travel vaccines

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Full Scope of Practice Services

Full Scope of Practice Services involve comprehensive patient consultations to assess, diagnose and treat (including prescribing medicines) authorised health conditions. The comprehensive and more complex nature of Full Scope of Practice services, requires the pharmacy to have:

- Pharmacist(s) with a Graduate Certificate in Pharmacist Prescribing (or other authorised training) as well as first aid and CPR certification
- Access to current and appropriate clinical references and resources
- Fit-for-purpose and well-maintained medical equipment and diagnostic tools
- A consultation room suitable for private and confidential patient discussions and examinations
- Policies, procedures and protocols to ensure consistent service delivery
- The ability to issue physical or electronic prescriptions
- A system to record detailed clinical notes of the care provided, and reporting to other healthcare providers as required
- Infection control protocols
- Referral pathways to other healthcare providers and services
- A formal process for collection, management and submission of patient reported experience measures (PREMs) and patient reported outcomes measures (PROMs)
- Regular case-based discussions with peers as a form of clinical practice review
- Service evaluation processes to review incidents, feedback, clinical records and jurisdictional requirements to improve the safety and quality of full scope of practice services

Examples of Full Scope of Practice services include, but are not limited to:

- Initiating hormonal contraception
- Diagnosis and treatment of acute nausea and vomiting associated with gastroenteritis
- Diagnosis and treatment of acute otitis media
- Diagnosis and treatment of asthma
- Lipid management
- Blood glucose management

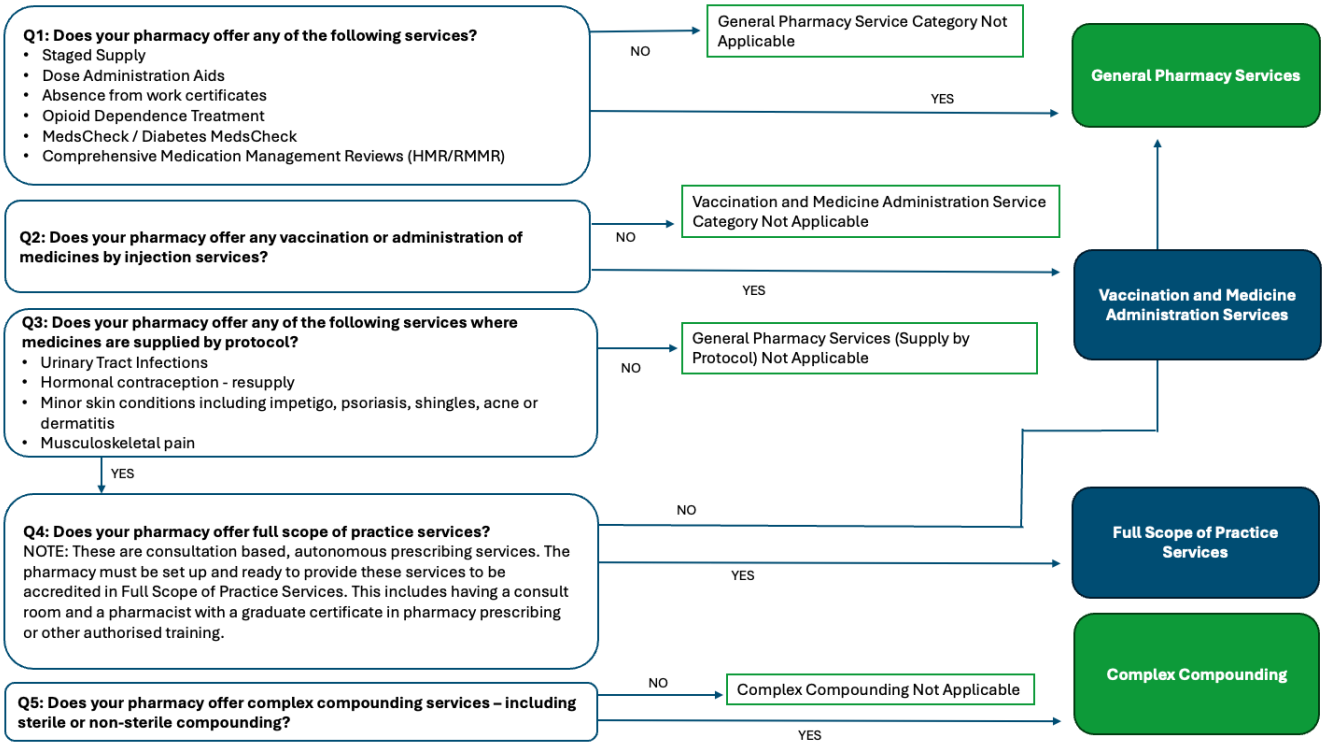
Complex Compounding

The delivery of complex compounding services, including sterile and non-sterile compounding of medicines, requires special competencies, equipment, processes and dedicated facilities. Risk assessments need to be conducted to ensure patient and staff safety, and robust quality assurance protocols need to be implemented to ensure consistency and efficacy of compounded medicines. To deliver these complex compounding services safely, pharmacies are required to:

- Have appropriately trained and qualified staff preparing compounded medicines
- Implement operating procedures to ensure consistent compounding practices
- Have access to current and appropriate references and resources
- Have a dedicated facility and equipment for complex compounding, and a separate dedicated cleanroom for sterile compounding (if undertaken)
- Conduct risk assessments to ensure the safety of staff working at the pharmacy, as well as patients who are prescribed a compounded medicine
- Conduct final product potency testing
- Package, label and store compounded medicines appropriately
- Assign beyond use by dates for compounded medicines
- Provide information to patients on the correct use and risks of compounded medicines
- Keep detailed records of compounding activities
- Evaluate compounding processes, incidents and feedback to improve the safety and quality of complex compounding services

The following Service Category Decision Tree can be used to assist pharmacies with understanding the service categories they should be assessed against under the QSPP.

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Community pharmacies are responsible for understanding and complying with any State-based regulatory requirements pertaining to the pharmacy services they deliver. The Quality & Safety Pharmacy Program requirements aim to define best practice in relation to pharmacy service delivery. Given this, the QSPP requirements may at times be more stringent than those required by some State and Territory Health Departments and pharmacies will be expected to meet the higher standard of practice.

While delivering pharmacy services, and providing medicines and advice to consumers, it is important that common clinical safety risks are identified and mitigated. Clinical safety areas in community pharmacies include medication safety, preventing and controlling infections, comprehensive and coordinated care, and recognising and responding to serious deterioration in patients. Many key tasks outlined in this Domain help ensure pharmacies have systems and processes in place to reduce the likelihood of clinical incidents that may cause harm to patients and pharmacy staff.

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2.1 Delivery of Pharmacy Services

Intention

Pharmacy services are delivered safely, through defined roles, appropriate training, regulatory compliance and patient feedback.

Outcome

The care I receive from my community pharmacy is safe and reliable, delivered by well-trained staff who listen to my feedback.

Standard Subclause

The pharmacy shall have systems to ensure programs and services are provided effectively, efficiently and in compliance with professional standards and guidelines, whether supplied within or external to the pharmacy.

2.1.1.1 The pharmacy shall ensure that the staff delivering each program or service are appropriately trained and/or qualified, including that they:

a. Can demonstrate competency, or the clinical knowledge required, to deliver each service.

b. Complete externally provided, Government-endorsed or accredited training where it is available and applicable.

c. Complete training on the pharmacy's procedures for the delivery of individual programs or services.

d. Document completion of the training they have undertaken.

Note:

Training must be provided to pharmacy staff, not just pharmacists, who are involved in the delivery of pharmacy services. As an example, pharmacy technicians, interns or students may be involved in the delivery of blood pressure monitoring or dose administration aid services and would require appropriate training as outlined above.

2.1.1.2 The pharmacy shall provide access to relevant and current reference material for each program or service it offers.

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2.1.1.3 The pharmacy shall ensure it provides a facility or area for the delivery of pharmacy programs or services, and that this facility or area is:

a. Fit for purpose, including the provision of adequate space, light and temperature.
b. Compliant with any legislative or professional practice requirements, including privacy legislation.
c. Cleaned to the level expected of a clinical healthcare destination.
d. Maintained to ensure it remains fit for purpose.

Note:

Maintenance of a facility or work area may include, but is not limited to, regular servicing of air-conditioning or heating units, inspection and repair of faulty water taps, chairs or privacy screens and replacing light bulbs. Where required, maintenance should be conducted by an appropriately qualified person. Documentation should be kept of maintenance works completed.

2.1.1.4 The pharmacy shall provide any equipment required for a program or service and ensure it is:

a. Fit for purpose.
b. Calibrated, if required.
c. Maintained as specified by the manufacturer.

Records of calibration and maintenance shall be kept by the pharmacy.

If external certification of equipment is required, records of this shall also be kept.

2.1.1.5 The pharmacy shall develop and implement a system for the consistent delivery of each program or service that is:

a. Compliant with any legislation, professional practice guidelines, funding or contractual requirements.
b. Provided to all staff members involved in the delivery of each program or service.
c. Reviewed at least annually and updated as required.

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Standard Subclause

2.1.1.6 The pharmacy shall have processes to ensure pharmacists deliver comprehensive care applicable to the services being delivered, by:

a. Conducting a risk screening and assessment.

b. Conducting a clinical assessment and diagnosis as appropriate.

c. Identifying the patient's goals of care.

d. Developing and agreeing a plan for care in partnership with the patient.

e. Delivering comprehensive care in accordance with the agreed plan for health care.

f. Collaborating and communicating with the patient's other healthcare providers as required.

g. Recalling patients for follow-up health care when required.

h. Reviewing and improving the processes of comprehensive care delivery.

i. Receiving a current advance care plan (where one is in place) and incorporating it into a patient's healthcare record.

Note:

Pharmacists should use their professional judgement to implement the required comprehensive care elements based on the patient's circumstances and the services offered by the pharmacy.

2.1.1.7 The pharmacy shall ensure it has adequate staff available to deliver pharmacy programs and services in addition to other pharmacy operations (such as dispensing prescriptions). The pharmacy shall have a process for prioritising activities if there is a reduction in the number of staff.

2.1.1.8 The pharmacy shall have communication processes to help patients maximise their attendance at planned appointments.

2.1.1.9 The pharmacy shall develop and implement a system to record each episode of care delivered, ensuring that:

a. The records are appropriate for the service that is delivered.

b. The recording occurs in a consistent manner.

c. The records are securely stored and in compliance with legislation.

d. The nature of the records is compliant with relevant professional practice guidelines, funding or contractual requirements.

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2.1.1.10 The pharmacy shall develop and implement a process to evaluate the programs and services delivered, including:

a. Monitoring the safety of the programs and services delivered.

b. Assessing the impact of programs or services, where possible.

c. Improving the programs and services based on the outcomes of the evaluation process.

Note:

The extent of the evaluation process may differ between programs and services.

Reflective Questions

- How do we ensure that all staff involved in delivering pharmacy services are appropriately trained and qualified?
- What strategies and mechanisms do we have in place to ensure staff work within their scope of practice?
- How do we provide easy access to reference materials to support the delivery of pharmacy services? How do we ensure the most current versions of the resources are being used by pharmacy staff?
- Is the location where pharmacy services are delivered consistently maintained to meet professional and legislative requirements?
- What processes are in place to ensure the proper calibration and maintenance of equipment needed for service delivery?
- How effectively do our systems and processes support the consistent delivery of pharmacy programs and services?
- In what ways do we collaborate with patients and other healthcare providers to ensure comprehensive care delivery?
- How do we monitor and address risks that may arise during the delivery of pharmacy services?
- Are there sufficient staff members available to deliver both pharmacy services and other pharmacy activities? What contingency plans exist for staffing shortages?
- Do our communication processes effectively encourage patients to attend planned appointments?
- Are the details of delivered pharmacy services accurately recorded? Are these records securely stored and compliant with legislative and professional guidelines?
- What methods do we use to evaluate the safety and impact of the services we provide, and how frequently do we update our processes based on evaluation outcomes?

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Key Tasks

- Review and comply with legislative and professional requirements that govern the delivery of pharmacy services, including:
  - **Training and education requirements**
  - **Premises requirements**
  - **Governance and staffing**
  - **Equipment and resources**
  - **Privacy regulations**
  - **Protocols (e.g. cold chain, emergency response, adverse events)**
  - **Record keeping and reporting.**
- Allocate an appropriate area of the pharmacy (e.g. a consultation room) that is compliant and appropriate for the delivery of pharmacy services, including consideration of access to equipment, patient privacy, lighting, temperature, adequate space, and cleanliness.
- Ensure the staff members that are delivering pharmacy services have the appropriate qualifications, training, competencies, and clinical knowledge to safely perform the pharmacy service.
- Identify and source relevant and current clinical references, evidence-based protocols and other resources to support the delivery of pharmacy services. These references should be located in an area that is easily accessible for pharmacy staff, to encourage their use.
- Schedule regular maintenance activities (e.g. air-conditioning units, chairs, privacy screens, lights) to ensure the area where pharmacy services are delivered is well maintained and has functional equipment.
- Ensure all medical devices or equipment are maintained, calibrated and certified (if applicable) in line with the manufacturer's recommendations.
- Develop systems and procedures for the consistent and safe delivery of pharmacy services. Procedures need to be kept in an easily accessible location and reviewed regularly to ensure they remain contemporary and compliant with legislation and standards of practice. Train relevant staff on the pharmacy's procedures for delivering pharmacy services.
- Adopt a comprehensive approach to screening patients at risk of certain health conditions (e.g. under expanded scope of practice services) providing clinical assessments and collaboratively developing care plans and treatment goals. Where appropriate, communicate and collaborate with other healthcare providers to ensure continuity of care.
- Establish robust systems to recall patients for follow-ups, when necessary.
- Maintain appropriate rosters to ensure adequate staffing for all pharmacy services and activities. Develop contingency plans or ways to prioritise pharmacy activities during times of staff shortages.
- Implement a booking system for patients and consumers to schedule pharmacy service appointments. Embed processes to send notifications, and appointment reminders (e.g. via text message, phone call or email).
- Establish and follow a process for accurately and consistently recording each occasion a pharmacy service as it is delivered, ensuring all pharmacy staff understand how to effectively record health information.
- Securely store patient records for the required duration as outlined in legislation.
- Develop a process for sharing service information as required (e.g. with other healthcare providers or for funding purposes).

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- Review the pharmacy's incident register at least annually for any incidents related to pharmacy services. Identify and implement improvements as appropriate.
- Gather and review feedback from patients and consumers (e.g. patient experience measures) to evaluate and assess the impact of pharmacy services, using evaluation data to make improvements to pharmacy services as appropriate.
- Conduct regular clinical audits of delivered pharmacy services to monitor both safety and effectiveness.

Monitoring and Evidence Examples

- Register of completed qualifications and/or training for all pharmacy staff involved in delivering services, including pharmacy technicians, dispensary technicians, dispensary assistants, pharmacy assistants, and interns.
- Demonstrated evidence that relevant and current references are easily accessible to pharmacy staff, including active links where electronic versions are being used.
- Demonstrated evidence that pharmacy services are delivered in a suitable location in the pharmacy that meets professional, privacy, and legislative requirements (including State and Territory requirements).
- Maintenance schedules and logs for facilities and equipment (e.g. air-conditioning units, chairs, privacy screens, lights) in areas where pharmacy services are delivered.
- Calibration, certification and maintenance schedules and logs for medical devices and equipment (such as blood glucose monitors, weighing scales and blood pressure machines).
- The pharmacy's procedures and/or protocols for the delivery of pharmacy services, and evidence they are kept in an easily accessible location.
- Documented care plans or goals of care developed during the course of pharmacy service delivery.
- Records of collaboration, communication, or referral to other healthcare providers.
- Screening and risk assessment tools used to identify patients at risk of disease.
- The pharmacy's staff roster.
- The pharmacy's booking system for service appointments and examples of notifications and appointment reminders.
- Records of pharmacy service delivery.
- Evidence the records are securely stored for the required legal period.
- Records of patient feedback and evaluation activities (e.g. surveys, the pharmacy demonstrates use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures) as well as any improvements made to the delivery of pharmacy services.
- Evidence of clinical audits conducted by pharmacists.
- The pharmacy's incident register related to the delivery of pharmacy services and evidence of actions taken.

Resources and Information

Pharmaceutical Society of Australia

- National Competency Standards Framework for Pharmacists in Australia 2016
- Professional Practice Standards 2023 - Version 6
- Professional Practice Guidelines

Australian Commission on Safety and Quality in Health Care

- Implementing Patient Experience / Patient Reported Experience Measures (PREMs)
- Patient-Reported Outcome Measures (PROMs)

Australian Government Office of the Australian Information Commissioner

- Australian Privacy Principles

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2.2 Prescribing by Pharmacists

Intention

Appropriately qualified community pharmacists prescribe safe and appropriate medicines and continually strive to improve prescribing practices.

Outcome

I receive safe and appropriate medication prescribed by a qualified pharmacist who adheres to therapeutic standards and professional guidelines and continuously improves their practice.

Standard Subclause

The pharmacy shall have systems to support the safe and appropriate prescribing of medicines by pharmacists in compliance with professional standards and guidelines.

2.1.2.1 The pharmacy shall maintain and follow a system for prescribing medicines when appropriate and within the scope of practice for a pharmacist.

2.1.2.2 Prescribing services offered at the pharmacy shall comply with relevant professional, legislative and jurisdictional requirements, and support the safe and appropriate use of medicines.

2.1.2.3 The pharmacy shall provide access to, and promote the use of, current, relevant, evidence-based Australian therapeutic guidelines and resources to support prescribing by pharmacists and as required by prescribing protocols or guidelines.

Note:

The pharmacy should provide access to additional resources relevant to the therapeutic goods prescribed by pharmacists working in the pharmacy—for example, antimicrobial stewardship standards to support the appropriate use of antimicrobials.

2.1.2.4 The pharmacy shall provide mechanisms for pharmacists to educate consumers about risks and benefits of, and alternatives to, prescribed medicines for their condition.

2.1.2.5 In compliance with relevant Codes of Conduct, the pharmacy shall have a process for declaring conflicts of interests that could affect or could be perceived to affect patient care and shall act in the best interests of patients.

2.1.2.6 The pharmacy shall have a process to evaluate the prescribing services delivered at the pharmacy, including:

a. Monitoring the safety of the prescribing.

b. Compliance with relevant Australian therapeutic guidelines.

c. Assessing the impact of the prescribing, where possible.

d. Improving prescribing services based on the outcomes of the evaluation process where possible.

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Reflective Questions

- How does the pharmacy ensure that prescribing practices align with current therapeutic guidelines and professional standards? Are there gaps?
- What processes does the pharmacy have in place to evaluate the safety and appropriateness of prescribing services provided?
- Can the pharmacy demonstrate compliance with relevant legislative and jurisdictional requirements for prescribing pharmacists?
- How accessible and up to date are the resources the pharmacy provides to support pharmacists in safe and informed prescribing? Are additional materials needed?
- In what ways do we educate patients and consumers about the risks, benefits, and alternatives associated with prescribed medications?
- How transparent are our prescribing services and are there mechanisms in place to identify and address conflicts of interest that may impact patient care?
- How do we monitor and improve the impact of prescribing services on patient outcomes and pharmacy operations?
- Are our pharmacists adequately trained and supported to comply with antimicrobial stewardship standards and other relevant prescribing protocols?
- What steps are taken to evaluate and adapt our prescribing services based on feedback and outcome assessments?
- How often do we reflect on our prescribing practices to identify areas for innovation, enhancement, or improved compliance?

Key Tasks

- Develop and embed systems and/or protocols to support the safe and appropriate prescribing within the pharmacist's scope of practice.
- Check that pharmacists have completed the necessary qualifications and training required to deliver prescribing services.
- Provide pharmacists with easy access to current, relevant, evidence-based therapeutic resources (e.g., Australian Therapeutic Guidelines, antimicrobial stewardship standards) and prescribing protocols to inform prescribing practices. Support pharmacists to incorporate established, evidence-based principles into their prescribing practices.
- Access appropriate consumer resources to inform and educate patients and consumers about the risks and benefits of, and alternatives to, prescribed medications.
- Recognise potential conflicts of interest that may arise for pharmacists delivering prescribing services. Develop processes for declaring and addressing conflicts of interest in compliance with relevant Codes of Conduct and Ethics to ensure pharmacists act in the best interests of patients when giving advice, suggesting treatments or making referrals.
- Capture incidents related to prescribing by pharmacists in the pharmacy's incident register. At least annually, review incidents to identify areas for improvement and document any changes made.
- At least annually, evaluate prescribing services to monitor prescribing safety and to assess the impact and effectiveness of the pharmacy's prescribing services. Feedback and outcomes should be evaluated and inform improvements to prescribing practices.
- Conduct periodic clinical audits to ensure ongoing compliance of pharmacists with legislative requirements, therapeutic guidelines and protocols.

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Monitoring and Evidence Examples

- Documented evidence (e.g. a compliance audit) that the pharmacy has reviewed the legislation, jurisdictional requirements and professional guidelines that apply to prescribing by pharmacists.
- The pharmacy's procedure(s) for the delivery of prescribing services and evidence they are kept in an easily accessible location.
- Records of completed qualifications and/or training for all pharmacists who deliver prescribing services.
- Demonstrated evidence pharmacists are practicing within their scope of practice for which they are appropriately qualified.
- Copies of current, relevant, evidence-based Australian Therapeutic Guidelines, prescribing protocols and resources applicable to the medicines potentially prescribed and evidence they are kept in an easily accessible location.
- Examples of patient education materials covering the risks and benefits of prescribed medicines, the rationale not to prescribe a medicine, or alternative treatment options.
- Practical evidence that the patient is informed of any interests or perceived interests in instances where the prescribing pharmacist is also offering dispensing services, this may include patient information sheets, advisory posters, or documented evidence in clinical notes.
- Records of patient feedback and evaluation activities (e.g. surveys, use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures) as well as any improvements made to prescribing services.
- Records of clinical audits completed against legislative requirements, therapeutic guidelines and protocols.
- Documented incidents in the pharmacy's incident register and identified improvements made to prescribing services (as applicable).

Resources and Information

Australian Health Practitioner Regulation Agency (AHPRA)

- Health Practitioner Shared Code of Conduct

Pharmaceutical Society of Australia

- National Competency Standards Framework for Pharmacists in Australia 2016
- Professional Practice Standards 2023 - Version 6
- Code of Ethics for Pharmacists
- Professional Practice Guidelines
- State and Territory Legislation for Pharmacists

Therapeutic Goods Administration

- The Poisons Standard (SUSMP)

Australian Commission on Safety and Quality in Health Care

- Antimicrobial Stewardship Clinical Care Standard
- Implementing Patient Experience / Patient Reported Experience Measures (PREMs)
- Patient-Reported Outcome Measures (PROMs)

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2.3 Safe and Appropriate Provision of Medicines, Medical Devices and Poisons

Intention

Medicines, medical devices, and poisons are provided appropriately through adherence to legislative requirements, professional standards, and adopting best practice in medicine management.

Outcome

I know the risk of medication errors is minimised as my community pharmacy has robust governance processes in place to manage medicines, medical devices and poisons.

Standard Subclause

The pharmacy systems shall be designed to ensure the most appropriate therapeutic goods are recommended and that consumers are provided with sufficient information to use the medicines safely and effectively. Therapeutic goods (including compounded medicines) and poisons shall be provided in accordance with legislative requirements, professional standards and guidelines whether supplied within or external to the pharmacy.

2.2.2.1 The pharmacy shall maintain and follow a system for dispensing prescribed medicines.

2.2.2.2 To support safe and appropriate use, the pharmacy shall comply with professional requirements, manufacturer's instructions and legislative and jurisdictional requirements for the supply of medicines including:

a. Non-prescription medicines.
b. Medicines that require special consideration or are subject to misuse.
c. Medical devices.
d. Poisons and other high-risk products.
e. Medicines packaged in dose administration aids.

2.2.2.3 The pharmacy shall have processes to ensure pharmacists work within their scope of practice to:

a. Take a detailed medication history on presentation or as early as possible in the episode of care.
b. Ensure a patient's medicines-related information is included in a patient's healthcare record.
c. Partner with patients and consumers in the management of their medicines.
d. Support patients and consumers to maintain a current and accurate medicines list.
e. Encourage patients to share their medicines list with other healthcare providers involved in their care and/or does so on a patient's behalf with their consent.
f. Use information on a patient's medication history to minimise risks in the planning and delivery of health care (for example, provision of clinical advice, referral for a medication review or a dose administration aid).

Note:

Pharmacists may apply a risk-based approach when taking and documenting a medication history as appropriate for each consumer's presentation.

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Standard Subclause

2.2.2.4 The pharmacy shall have processes to ensure pharmacy staff provide information on medicines tailored to the patient's needs and preferences and most appropriate for the medicine being provided.

2.2.2.5 The pharmacy shall maintain and follow a system for simple compounding.

2.2.2.6 The pharmacy shall maintain and follow a system for the management of medication-related problems, which includes:

a. Identifying medication-related problems which may include, but are not limited to, adverse drug reactions, dispensing errors (including near miss events), drug interactions and inappropriate care or advice.

b. Performing an intervention.

c. Documenting the problem and the intervention made.

d. Reviewing medication-related problems and interventions made to identify whether changes in clinical practice or processes are required.

e. Taking action to minimise potential medication-related problems.

f. Reporting medication-related problems as required (for example, suspected adverse drug reactions to the Therapeutic Goods Administration, manufacturer, funding provider or other healthcare providers).

2.2.2.7 The pharmacy shall have a system to collaborate with other health care providers to resolve problems relating to medicine use. Details of the collaboration, or attempts to collaborate, shall be documented.

2.2.2.8 The pharmacy shall maintain and follow a system for stock pricing which:

a. Ensures stock prices are consistent with the planned pricing structure and any advertised price.

b. Ensures any displayed or advertised stock price is consistent with the price charged at the point of sale.

2.2.2.9 The pharmacy shall have an effective and efficient process for processing pharmacy sales, including returns and refunds.



Reflective Questions

- How consistently do we document medication histories and ensure they are integrated into patients' healthcare records?
- In what ways are we engaging patients in the management of their medicines, and how effective are these practices in encouraging patient adherence?
- What strategies are in place to ensure pharmacy staff have the knowledge and skills to provide non-prescription medicines, high-risk products, medical devices, and dose administration aids safely and appropriately?
- What processes are used to identify patients at risk of medicine-related problems or adverse events?
- Are our processes for identifying and addressing medication-related problems adequate, and how could they be improved?
- How do we ensure that information provided to patients about their medicines is tailored to their unique needs and preferences?
- What strategies are in place to support patients and carers to raise concerns about medicine-related risks and clarify the information they are provided?
- How do we obtain and document accurate medication histories and when do we consider this to be necessary?
- How are patients encouraged to keep their own medicine list and to share this with other relevant healthcare providers?
- How effective is our system for simple compounding, and are there areas where safety or efficiency could be enhanced?
- Do we have a clear and consistent approach to pricing and advertising therapeutic goods, and does this align with consumer expectations and transparency and legislative requirements?
- How robust are our collaborations with other healthcare providers, and what strategies can strengthen these partnerships?
- How is informed consent incorporated into our processes for sharing a patient's medicines information and/or medicines list with other relevant healthcare providers on their behalf?
- How do pharmacy staff identify and record the details of other healthcare providers involved in a patient's care?
- Are all interventions related to medication-related problems well-documented, and how do we review these for opportunities to improve practices?
- What mechanisms are in place for reporting suspected adverse drug reactions, and are these systems utilised effectively by staff?



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Key Tasks

- Develop and embed a dispensing system that ensures prescription medicines are supplied safely, accurately and efficiently.
- Establish and maintain a process for supplying non-prescription medicines to patients and consumers that complies with professional, legislative, and jurisdictional requirements, and supports pharmacy staff to consistently follow the defined processes.
- Ensure pharmacy staff undertake regular training to support the supply of over-the-counter medicines. This includes:
  - **Initial training to support the supply of Pharmacy Medicines and Pharmacist Only Medicines, commenced within the first 6 months of employment. Examples of appropriate initial training include:**
  - **SIRCIND002 Support the supply of Pharmacy Medicines and Pharmacist Only Medicines or its successor, or**
  - **where staff are ineligible to complete this training, training that is to the same standard, delivered by an RTO, and results in a Statement of Completion.**
  - **A minimum of 3 hours of approved annual refresher training to retain contemporary knowledge relevant to the supply of over-the-counter medicines, minor health conditions and/or supporting pharmacists to deliver full scope of practice services (e.g. taking pre-consultation vitals such as temperature and blood pressure).**
- Establish and maintain a consistently applied process for the supply of Pharmacist-Only Medicines, and medicines that require special consideration or are subject to misuse, that ensures a pharmacist is directly involved, and details of the supply (or refusal to supply) are recorded as appropriate.
- Implement a system to ensure medical devices supplied by the pharmacy are listed on the Australian Register of Therapeutic Goods and consumers are given adequate instruction on their appropriate use.
- Implement a system for the safe and appropriate provision of poisons. The system should ensure the safe storage of poisons and chemicals of concern, ascertaining the intended use prior to supply, counselling on proper use and disposal of poisons, appropriate packaging and warning labels, and recording details of the supply as required.
- Establish and embed a consistently applied process to ensure medicines packaged in a dose administration aid have been checked and signed off by a pharmacist, are provided to the correct patient, and accompanied with suitable counselling and information on its correct use.
- Gather a detailed medication history when appropriate for a patient's presentation and healthcare needs.
- When considered appropriate, support patients to maintain an accurate medication list and encourage them to share it with their other healthcare providers.
- Provide pharmacy staff with access to current and reputable medicines information that can be readily provided to patients and consumers. The information provided, and its format, should be tailored to the needs and preferences of individual patients.
- Maintain and follow a procedure for the simple compounding of medicines which covers who can prepare the compounded products, the quality of ingredients to be used, where compounding is to be conducted, maintenance of the equipment to be used, labelling and recording of compounded products.

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Key Tasks cont.

- Establish and embed a process for the timely identification, management, recording and analysis of medication-related problems which include, but are not limited to, adverse drug reactions, dispensing errors (including near miss events), drug interactions and inappropriate care or advice.
- Routinely review medication-related problems and interventions and make changes to clinical practice or processes as required. Document improvements or actions taken to reduce the likelihood of further medication-related problems.
- Establish and maintain a system for reporting medication-related problems as required (e.g. suspected adverse drug reactions to the Therapeutic Goods Administration, manufacturer, funding providers other healthcare providers). Support pharmacy staff to follow the process consistently.
- Collaborate with other healthcare providers to share information and resolve medicine-related problems (including potential problems) for patients. Document each collaboration (or attempt to collaborate) and embed a process for obtaining patient consent before sharing their medication information.
- Implement a pricing policy for the pharmacy to ensure a consistent approach to pricing stocked items. Pricing of medicines should be transparent and consistent, ensuring advertised prices match the point-of-sale prices.
- Implement efficient systems for processing sales, including handling returns and refunds.

Monitoring and Evidence Examples

- The pharmacy has a range of procedures to support safely providing medicines, which may include:
  - **A dispensing procedure which includes barcode verification processes,**
  - **the provision of non-prescription medicines,**
  - **the provision of Pharmacist-Only Medicines and medicines that require special consideration or are subject to misuse,**
  - **ensuring medical devices provided by the pharmacy are listed on the Australian Register of Therapeutic Goods and supplied with appropriate instructions for use,**
  - **the provision of poisons,**
  - **the provision of dose administration aids (DAAs) and evidence the processes are consistently followed by pharmacy staff.**
- Incidents relating to dispensing are documented in the pharmacy's incident register and include details of corrective actions taken to prevent future incidents.
- Records of qualifications and training completed by pharmacy staff to support the provision of Pharmacy Medicines and Pharmacist Only Medicines, including:
  - **Completion of SIRCIND002 Support the supply of Pharmacy Only and Pharmacist Only Medicines or its successor, or where staff are ineligible to complete this training, training that is to the same standard, delivered by an RTO and results in a Statement of Completion (or if employed within the last 3 months enrolment in the course and working towards completion).**
  - **Completion of Certificate II, III or IV in Community Pharmacy (or if employed within the last 6 months enrolment in Certificate II, III or IV in Community Pharmacy and working towards completion).**
  - **A minimum of 3 hours of approved annual refresher training that is contemporary and relevant to the supply of Pharmacy Medicines and Pharmacist Only Medicines and/or related minor health conditions.**

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- Evidence of adverse drug reactions or allergies recorded in a patient's healthcare record.
- Template medication lists for patients.
- Evidence of the pharmacy's process for providing patients with a medication list (e.g. from the dispensing software).
- Demonstrated evidence of how medicines information is accessed, tailored and provided to patients.
- Evidence of the pharmacy's simple compounding procedure.
- Records of compounded medicines.
- Demonstrated evidence of the area and equipment used for compounding, as well as the ingredients including where they are stored and their expiry.
- Records of medication-related problems (e.g. in the pharmacy's incident register or patient healthcare record) including actions taken and improvements made to clinical practice or processes as required.
- Evidence that medication-related problems have been reported as required (e.g. to the Therapeutic Goods Administration or manufacturer).
- Records of collaboration, or attempts to collaborate, with other healthcare providers to resolve medication-related problems.
- The pharmacy's pricing policy.
- Evidence of the pharmacy's system (e.g., procedure/checklist/ policy/protocol) for processing pharmacy sales, returns and refunds supported by records of transactions, returns, refunds and training of pharmacy staff involved in the process.

Resources and Information

Pharmaceutical Defence Limited (PDL)

- Guide to Good Dispensing
- Risk Management Guide for Pharmacists

Therapeutic Goods Administration

- Australian Register of Therapeutic Goods

Australian Commission on Safety and Quality in Healthcare

- Medication reconciliation
- Resources for obtaining a best possible medication history

National Aboriginal Community Controlled Health Organisation

- Medicines Information Co-Designed with NACCHO
- Medicines List Template

Other patient information

- Translating and Interpreting Service (TIS)

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2.4 Complex Compounding

Intention

Compounded medicines are made in an appropriate facility, with appropriate equipment, high quality ingredients and with adherence to stringent guidelines, to promote effective, consistent, quality medicines tailored to individual patient needs.

Outcome

I receive medicines made with high-quality ingredients and prepared under strict guidelines, ensuring they are effective and consistent in quality.

Standard Subclause

The pharmacy shall have systems to ensure that the efficacy, safety and stability of all products prepared in the pharmacy are provided in accordance with legislation, professional standards and guidelines.

2.2.3.1 The pharmacy shall ensure there are appropriately trained and/or qualified staff to prepare compounded medicines, including that they:

a. Can demonstrate competency, or the clinical knowledge required, to prepare compounded medicines.

b. Complete externally provided training where it is available.

c. Complete training on the pharmacy's procedures for the preparation of compounded medicines.

d. Document completion of the training they have undertaken.

Note:

Training must be provided to all pharmacy staff, not just pharmacists, who are involved in the preparation of compounded medicines.

2.2.3.2 The pharmacy shall ensure they provide access to operating procedures and relevant reference material for all compounds prepared.

2.2.3.3 The pharmacy shall ensure there is a dedicated facility and equipment for complex compounding and a separate dedicated cleanroom for sterile compounding if undertaken.

2.2.3.4 The pharmacy shall maintain and follow a system of risk assessment for workplace health and safety for staff working in the compounding facility.

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Standard Subclause

2.2.3.5 Each time a medicine is compounded, the pharmacy shall follow a system of risk assessment to ensure:

a. Appropriate circumstances for compounding.

b. Patient safety (including taking a medication history).

c. Product formulation.

d. Product quality.

2.2.3.6 The pharmacy shall source high-quality raw materials (pharmaceutical grade where applicable) from reputable suppliers.

2.2.3.7 The pharmacy shall maintain and follow a system to identify, handle, store and dispose of materials safely, both raw and compounded.

2.2.3.8 The pharmacy shall maintain and follow a system to ensure consistency of compounded products, including appropriate instructions for compounding and a system for final product potency testing.

2.2.3.9 The pharmacy shall maintain and follow a system for appropriate packaging, labelling and storage of finished compounded products.

2.2.3.10 The pharmacy shall implement and follow a process for assigning beyond use by dates to ensure efficacy and safety of compounded medicines.

2.2.3.11 The pharmacy shall follow a process for the provision of relevant counselling to patients on the correct use and risks of compounded medicines.

2.2.3.12 The pharmacy shall maintain and follow a system of initiating, managing and reporting a product recall of compounded products.

Reflective Questions

- How might the pharmacy improve the current systems around safety, efficacy, and consistency of complex compounded medicines?
- What strategies have been implemented to maintain and ensure the proper upkeep of facilities and equipment dedicated to complex compounding?
- How does the pharmacy facilitate appropriate staff training to ensure competency and currency of knowledge in complex compounding practices?
- What processes are in place to ensure staff are consistently following the pharmacy's procedures for preparation of compounding and risk assessment?
- What are the key factors the pharmacy considers when selecting compounding suppliers?
- What are the key considerations of raw materials that we need to consider to comply with safety standards?
- How can procedures be designed to consistently ensure the potency and quality of compounded products?
- What additional processes do we need to implement to receive, store, handle and dispose of hazardous drugs and materials appropriately?

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- What methods or practices are implemented to conduct effective testing of final products for quality control? How are we prioritising testing of high-risk formulations (e.g. paediatric medicines, drugs with a narrow therapeutic index) and how do we determine the frequency of testing?
- What approaches might be most effective in counselling patients on the correct use, storage, disposal, adverse events and other risks related to compounded medicines?
- How does the pharmacy ensure labelling and packaging practices are aligned with official regulatory standards?
- How does the pharmacy determine the compatibility of all ingredients with the packaging material used for compounded products?
- How does the pharmacy manage and report adverse events related to compounded medicines?
- What systems or protocols are in place to manage, and report recalls of compounded products efficiently?
- How does the pharmacy ensure the safety and efficacy of a compounded medicine for a specific patient?
- What protocols and resources does the pharmacy use to substantiate the expiry date (beyond-use dates) that it assigns to compounded medicines?

Key Tasks

- Facilitate access to externally provided, competency-assessed training to ensure all pharmacy staff involved in compounded medicines have the right qualifications, skills and knowledge for their role. Keep records of all training completed.
- Conduct regular competency assessments (at least annually) to evaluate staff knowledge and skills related to complex compounding procedures.
- Develop and provide easy access to detailed standard operating procedures and/or instructions for complex compounding to ensure the consistency, safety and quality of compounded products.
- Source relevant and current reference materials for all ingredients and compounded medicines prepared by the pharmacy, and ensure staff are able to easily access reference material.
- Maintain a designated, physically separate and fit-for purpose area for complex compounding, including a separate dedicated area for sterile compounding, as required.
- Develop and embed a procedure for cleaning and, where applicable, disinfecting, decontaminating and/or deactivating the area used for complex compounding.
- Identify equipment in the compounding facility that requires auditing, calibration, certification, and/or maintenance as specified by the manufacturer. Develop a schedule for the routine auditing, calibration, maintenance and/or certification of the equipment. Execute and document completion of these activities.
- Conduct a workplace health and safety risk assessment for all staff working in the compounding facility and implement steps to eliminate, mitigate or manage identified risks.
- Provide appropriate personal protective equipment for all staff involved in compounding medicines.
- Establish, maintain and follow a policy to minimise risk and monitor the health of pharmacy staff involved in compounding hazardous medicines including cytotoxic and hormone preparations.
- Provide chemical spill kits and ensure they are accessible to staff working in the compounding facility. Train staff working in the compounding facility on the correct use of the chemical spill kit.
- Follow and maintain a risk assessment system for each time a medicine is compounded to ensure appropriate circumstances for compounding, patient safety (by taking a thorough medication history), correct product formulation and product quality.
- Establish criteria to ensure suppliers of raw materials for compounding are reputable. Maintain a list of reputable suppliers for ordering purposes.

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Key Tasks cont.

- Establish criteria to ensure raw materials for compounding are of suitable quality.
- Implement systems for safe receipt, handling, storage, and disposal of raw and compounded materials.
- Establish and embed a system to ensure consistency, safety and quality of compounded products, including appropriate instructions for compounding.
- Establish a system for final product potency testing to ensure consistency, efficacy and quality. A final product potency test must be completed on two different drugs and two different dosage forms per compounding technician and compounding pharmacist per year.
- Implement a process to assign appropriate beyond use dates to guarantee product effectiveness and safety.
- Establish and embed processes for packaging and labelling compounded medicines that meet regulatory standards.
- Store compounded medicines under conditions that maintain their efficacy and stability.
- Implement a system to routinely provide patients with clear instructions and information on the correct use and risks associated with compounded medicines.
- Develop and maintain a system for initiating, managing, and reporting product recalls effectively.
- Implement systems to continually monitor compliance with legislation, standards and guidelines related to compounding, including advertising and third-party supply.

Note:

Where community pharmacies offer compounding services via a third party, systems and protocols need to be implemented to ensure appropriate risks assessments are conducted and counselling is provided to patients. Roles and responsibilities for both parties need to be defined and documented.

Monitoring and Evidence Examples

- Records and certificates of completed training for pharmacy staff involved in compounding medicines. Training records may be reflective of formal, externally provided courses, internal training on the pharmacy's procedures or ongoing training related to complex compounding.
- Evidence of training related to preparing hazardous compounded medicines, and sterile compounding for pharmacy staff involved in these compounding activities.
- Results of competency assessments (both training assessments and assessments of compounded products) for staff involved in compounding. For sterile compounding, this should include results of garbing and aseptic manipulation competency evaluations (e.g. visual observation, gloved fingertip and thumb sampling, media-fill test, surface sample of compounding area).
- The pharmacy's standard operating procedures (SOPs) for all compounding medicines.
- The pharmacy's instructions for compounding.
- Logs or forms used when preparing compounded medicines as required by the pharmacy's SOPs.
- Evidence that relevant reference materials, including safety data sheets, lists of ingredients and reputable master extemporaneous formulary databases, are easily accessible to pharmacy staff.

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- Patient Compounding Records specific for each product compounded for each patient.
- Documented evidence of each batch of compounded medicine (where batching is permitted).
- Demonstrated evidence that there is a dedicated, separate and fit-for-purpose facility for complex compounding, and a separate dedicated area, equipment and raw ingredients for sterile compounding, if provided.
- The pharmacy's process (e.g., procedure/checklist/policy/protocol) for cleaning and, where applicable, disinfecting, decontaminating and/or deactivating the area for complex compounding.
- Evidence of the pharmacy's auditing, calibration, maintenance and certification schedule applicable to the equipment used in the compounding facility.
- Records or logs of calibration and maintenance checks for compounding equipment.
- Evidence of accreditation of the pharmacy to relevant standards by an external agency if undertaken.
- Documented workplace health and safety risk assessments including details of how risks are eliminated, mitigated or managed. This should include staff exclusion policies (e.g. pregnancy, illness, injuries and wounds).
- Demonstrated evidence that appropriate personal protective equipment is available (e.g., laboratory coat, face mask, beard covers, disposable gloves, hair covers, shoe covers and protective eyewear).
- The pharmacy's policy for monitoring the health of staff who compound hazardous medicines including cytotoxic and hormone preparations. Evidence that the policy is followed.
- Demonstrated evidence that the compounding pharmacy has chemical spill kits in an accessible location for staff. Records of completed training by pharmacy staff related to the use of chemical spill kits.
- Documented evidence of risk assessments completed for patients presenting a prescription for a compounded medicine.
- Documented medication histories for patients.
- Protocols for identifying potential incompatibilities in compounded medications.
- The pharmacy's criteria for supplier selection, a register of reputable suppliers and supplier agreements.
- Inventory logs for all raw materials, including licenses or compliance certificates verifying their pharmaceutical-grade (e.g. TGA license, Good Manufacturing Practice compliance certification, compliance with an acceptable pharmacopoeia monograph). The pharmacy's processes for identifying, handling, storing and disposing of materials, including raw ingredients, stock dilutions and compounded medicines. Records of completed training by pharmacy staff related to the procedure above.
- Safety data sheets for all raw ingredients. Demonstrated evidence that raw ingredients are stored in accordance with safety data sheets (SDS) and labelled with expiry dates and batch numbers.
- Demonstrated evidence that hazardous raw ingredients are labelled and stored separately in a dedicated area to minimise risk and contamination.
- Documented evidence of quality verification measures used by the pharmacy such as organoleptic properties (e.g. texture, smell, consistency, appearance, colour, continuity, hardness, integrity).
- Evidence of the pharmacy's procedure for final product potency testing.
- Records of the outcomes from final product potency tests completed.
- The pharmacy's procedure for assigning beyond use by dates for compounded products.
- Records demonstrating adherence to beyond use dating protocols and demonstrated evidence beyond use dates are assigned to compounded products.

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- The pharmacy's procedure for packaging and labelling compounded products. Demonstrated evidence that the procedure is followed by pharmacy staff.
- Evidence that compounded products are labelled (including correct naming, instructions and expiry dates) and stored appropriately.
- The pharmacy's process for patient counselling, including evidence of the types of written information provided to patients and how staff handle enquiries.
- Demonstrated evidence that ancillary labels are applied to compounded medicines.
- The pharmacy's process for managing product recalls of compounded products.
- A recall log detailing actions taken and communications with affected patients.

Resources and Information

Pharmaceutical Society of Australia

- Professional Practice Standards Version 6
- National Competency Standards Framework for Pharmacists in Australia
- Australian Pharmaceutical Formulary

Pharmacy Board of Australia

- Guidelines on compounding of medicines

Pharmaceutical Inspection Co-operation Scheme

- Guide to Good Practices for the Preparation of Medicinal Products in Healthcare Establishments (PE 010)
- Guide to Good Manufacturing Practice for Medicinal Products (PE 009)

Safe Work Australia

- Chemicals search function



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2.5 Indirect Provision of Medicines

Intention

Patient safety, compliance with legal requirements, and professional obligations are upheld even when medicines are provided indirectly.

Outcome

I trust that even though I don't go to the community pharmacy to collect my order, I am confident they have effective systems for the safe provision of medicines.

Standard Subclause

When medicines are provided to consumers and patients without direct interactions with pharmacy staff, the pharmacy shall have systems to ensure that the safety of the patient is maintained and that legal and professional obligations are met.

2.2.4.1 The pharmacy shall maintain and follow a system for the indirect provision of medicines (including compounded medicines), medical devices, and poisons, including:

a. Ordering via internet, post, telehealth and fax.

b. Delivery by a third party on behalf of the pharmacy.

c. Ordering from, and delivery to, other medical facilities such as residential aged care facilities and Aboriginal community health organisations.

d. Product deliveries conducted by pharmacy staff.

A risk assessment shall be conducted for all indirect supply activities and pharmacist involvement (for example, counselling) is still required in line with professional and legislative obligations.

Note:

When entering contractual arrangements with third party providers, the pharmacy still holds the responsibility for the safe provision of medicines and must not breach legislative or professional requirements. In addition, contractual requirements with third parties (for example Aboriginal Community Controlled Health Organisations) should be reasonable and ethical to best serve patients and communities.

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Reflective Questions

- How do we ensure the safety of patients when medicines are provided indirectly (e.g. delivered by the pharmacy or a third-party, posted to the patient, collected from a Pharmacy Collection Unit (PCU) or purchased through a Community Pharmacy Vending Machine (CPVM))?
- What measures are in place to maintain legal and professional obligations during indirect provision?
- How effectively are our risk assessments for indirect supply activities conducted and implemented?
- Are pharmacists adequately involved in counselling patients receiving medicines through indirect channels?
- What systems or processes do we rely on to monitor and improve the quality of medicine delivery by third parties?
- Do our contractual arrangements with third-party providers align with ethical and professional standards?
- How do we address challenges specific to indirect delivery to facilities like aged care homes or Aboriginal community health organisations?
- Are our policies and procedures for indirect provision clear, detailed, and tailored to our pharmacy's unique context?
- What feedback mechanisms are in place to identify issues or improvements in our indirect supply process?
- How regularly do we review and update our indirect provision practices to ensure compliance with evolving legislative requirements and service delivery frameworks?

Note:

Community pharmacies retain full responsibility for safe medicine provision, even when relying on third-party providers. Policies and procedures should align with legislative and ethical standards to best serve patients and communities.

Key Tasks

- Establish and maintain a comprehensive procedure for the indirect provision of medicines, medical devices, and poisons.
- Implement processes for patients and consumers to order medicines through the most appropriate method(s) (e.g. internet, phone, post, telehealth, prescription apps).
- Implement processes for packaging medicines for indirect supply to ensure compliance with manufacturer's storage and delivery specifications are met.
- Develop clear third-party delivery processes which ensure compliance with professional and legislative obligations.
- Provide training and education to delivery personnel (including staff and third-party providers) on their obligations and the pharmacy's indirect supply procedures.
- Manage and coordinate tailored systems and procedures for facility-specific deliveries and orders (medical facilities, residential aged care homes and Aboriginal community health organisations).
- Conduct thorough risk assessments for all indirect supply activities to identify and mitigate potential risks effectively.
- Embed pharmacist involvement and participation in patient counselling, even for medicines delivered indirectly, in accordance with legal and professional standards.
- Ensure contractual arrangements with third party providers uphold safety and compliance requirements, protect patient data and intellectual property, and address ethical and professional contractual considerations.
- Maintain a system to track and record indirect supply activities conducted by the pharmacy.
- Implement monitoring and feedback mechanisms to monitor the quality of medicine delivery and gather feedback for continuous improvement.

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Monitoring and Evidence Examples

- Detailed policies and procedures for the indirect provision of medicines, medical devices and poisons.
- Risk assessments for indirect supply activities, including thorough evaluations of potential risks and mitigation strategies.
- Contractual agreements with third-party providers that align with ethical and professional standards and address patient data security and intellectual property.
- Evidence of training completed by delivery personnel (both staff and third-party providers) on the pharmacy's indirect supply procedures and their responsibilities.
- Records of pharmacist counselling provided during indirect supply of medicines.
- Logs of medicine deliveries, including delivery dates, methods, and recipients.
- Systems for tracking the quality of medicine delivery by third parties, such as consumer satisfaction surveys or delivery audits.
- Feedback forms or mechanisms to identify issues and suggest improvements to the indirect supply process.
- Records of incidents relating to indirect supply on the pharmacy's incident register.
- Documentation of engagement with facilities (e.g. aged care homes or Aboriginal community health organisations) to review and improve indirect supply processes.

Resources and Information

- Pharmacy Guild of Australia Position Statement – Vending Machines and Collection Units in Community Pharmacy
- Pharmacy Guild of Australia Position Statement - In-Person Supply of Medicines



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2.6 Preventing and Controlling Infections

Intention

Robust infection control measures reduce healthcare-associated infections and provide a safe, clean and hygienic environment for the delivery of healthcare.

Outcome

I know my community pharmacy values my safety through the infection control measures such as hand hygiene, respiratory etiquette, and vaccination protocols they have in place.

Standard Subclause

Evidence-based systems shall be used to prevent and control healthcare-associated infections. Patients presenting with, or with risk factors for, infection or colonisation with an organism of local, national or global significance shall be identified promptly and receive the necessary management and treatment. The pharmacy shall be clean and hygienic.

2.3.1.1 The pharmacy shall have processes to apply standard and transmission-based precautions that are fit for the setting and consistent with current infection control guidelines, jurisdictional requirements and relevant jurisdictional laws and policies, including work health and safety laws.

2.3.1.2 The pharmacy shall have a hand hygiene process that is incorporated in its overarching infection prevention and control program as part of standard precautions and:

a. Is consistent with the National Hand Hygiene Initiative and jurisdictional requirements.

b. Supports pharmacy staff and consumers to practise hand hygiene.

2.3.1.3 The pharmacy shall support staff and consumers to practise respiratory hygiene, cough etiquette and physical distancing.

2.3.1.4 Where aseptic technique is required as part of the provision of health care, the pharmacy shall have processes to:

a. Identify procedures where aseptic technique applies.

b. Monitor healthcare providers' practices to ensure compliance with the pharmacy's policies and procedures on aseptic technique.

Note:

Aseptic technique is a set of practices that protect patients and healthcare providers from health care-associated infections and from contact with blood, body fluid and body tissue. Examples of procedures where aseptic technique is required include, but are not limited to, wound dressings, sterile compounding and parenteral medication preparation.

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Standard Subclause

2.3.1.5 Where invasive medical devices are used, the pharmacy shall have processes for the appropriate use and management of invasive medical devices that are consistent with current infection control guidelines.

2.3.1.6 The pharmacy shall have processes to maintain a clean, safe, and hygienic environment—in line with current infection control guidelines, and jurisdictional requirements—to:

a. Respond to environmental risks, including novel infections.

b. Require cleaning and disinfection using products listed on the Australian Register of Therapeutic Goods consistent with manufacturers' instructions for use and recommended frequencies.

c. Provide access to training on cleaning processes for routine and outbreak situations and novel infections.

2.3.1.7 The pharmacy shall have processes to evaluate and respond to infection risks for:

a. New and existing equipment, devices and products used in the pharmacy.

b. Clinical and non-clinical areas and workplace amenity areas.

c. Maintaining, repairing and upgrading buildings, equipment, furnishings and fittings (where the pharmacy has authority to do so).

d. Handling, transporting and storage of linen (if applicable).

e. Novel infections and risks identified as part of public health response or pandemic planning.

2.3.1.8 The pharmacy shall have a risk-based pharmacy staff vaccine-preventable diseases screening and immunisation process that:

a. Is consistent with the current edition of the Australian Immunisation Handbook.

b. Is consistent with jurisdictional requirements for vaccine-preventable diseases.

c. Identifies and addresses specific risks to pharmacy staff, consumers and patients.

2.3.1.9 The pharmacy shall have risk-based processes for preventing and managing infections among pharmacy staff that:

a. Are consistent with the relevant State or Territory work health safety regulation and current infection control guidelines.

b. Align with State and Territory public health requirements for pharmacy staff screening and exclusion periods.

c. Manage risks to pharmacy staff, patients and visitors, including for novel infections.

d. Promote non-attendance or remote attendance at work where possible when infection is present or suspected. Where non-attendance is unavoidable, appropriate personal protective equipment (PPE) should be used.

e. Plan for, and manage, ongoing service provision during outbreaks and pandemics or events where there is increased risk of transmission of infection.

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Standard Subclause

2.3.1.10 Where reusable equipment, instruments and devices are used, the pharmacy shall have:

a. Processes for reprocessing that are consistent with relevant national or international standards, in conjunction with manufacturers' guidelines.

b. A process for critical equipment, instruments and devices that can identify:

   i. The patient.

   ii. The procedure.

   iii. The reusable equipment, instruments and devices that were used for the procedure.

c. Processes to plan and manage reprocessing requirements and additional controls for novel and emerging infections.

Reflective Questions

- How effectively are we implementing infection control measures, and what areas could be improved?
- What strategies do we have in place to ensure our hand hygiene processes support both staff and consumer compliance?
- Are our respiratory hygiene and physical distancing policies clearly communicated and accessible to all stakeholders (e.g. pharmacy staff, patients and consumers)?
- How consistently do we adhere to aseptic techniques in relevant procedures, and how do we monitor compliance?
- What systems are in place to evaluate, manage, and mitigate risks associated with invasive medical devices?
- Do our cleaning and maintenance protocols align with current infection control guidelines and address outbreak situations effectively?
- How do we assess the effectiveness of our vaccine-preventable disease processes for staff?
- What steps are taken to prevent and manage infections among staff, and how do these align with public health regulations and risk management protocols?
- Are our processes for reprocessing reusable equipment, instruments, and devices both reliable and compliant with national and international standards?
- How do we plan for and manage service provision during outbreaks, pandemics, or other infection-related emergencies?

Key Tasks

- Implement evidence-based systems to prevent and control healthcare-associated infections. Embed these systems into all relevant pharmacy policies and procedures.
- Apply standard and transmission-based precautions aligned with infection control guidelines and jurisdictional requirements.
- Provide equipment, supplies and products to pharmacy staff to minimise the risk of infection transmission (e.g. evidence-based cleaning products, face masks, tissues, hand sanitiser).

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Key Tasks cont.

- Establish and embed:
  - **Hand hygiene processes that support pharmacy staff and consumer compliance, consistent with the National Hand Hygiene Initiative.**
  - **Respiratory hygiene, cough etiquette, and physical distancing (when required due to outbreaks or pandemics) processes for staff and consumers.**
- Provide training to pharmacy staff related to hand hygiene processes, respiratory hygiene, cough etiquette, physical distancing and effective cleaning and maintenance protocols for routine and outbreak situations.
- Promote the implementation of hand and respiratory hygiene processes, and physical distancing (when necessary), through visual aids (e.g. such as posters, electronic displays, distancing markers).
- Identify tasks where aseptic technique is required (e.g. wound care, sterile compounding, parenteral medication preparation) and implement protocols to support correct aseptic technique. Train relevant pharmacy staff on correct aseptic technique and monitor their compliance with the pharmacy's protocols.
- If invasive medical devices are used in the delivery of pharmacy services (e.g. devices inserted through the skin), implement processes to support their proper use and management, and to minimise infection risks. Ensure pharmacy staff involved in the use of invasive medical devices have completed accredited training on how to insert such devices, as well as internal training on the pharmacy's processes.
- Implement a cleaning schedule to maintain a clean and hygienic environment consistent with infection control guidelines. This should include regularly cleaning and disinfecting patient interaction areas to uphold clinical healthcare standards.
- Develop a planned response to outbreaks of communicable diseases within the local community and implement strategies to reduce further spread of infection.
- Identify and evaluate infection risks in the pharmacy (e.g. dust, mould, waste materials, infectious agents, furnishings, equipment, linen) and implement processes to eliminate or reduce such risks.
- Embed processes to promptly identify and manage patients with infection risks or colonisation by significant organisms as required.
- Develop a policy to facilitate immunisation for pharmacy staff against vaccine-preventable diseases.
- Develop risk-based processes to prevent and manage acute infections among pharmacy staff, aligned with work health safety regulation and public health requirements.
- Identify equipment, instruments, and devices used in the pharmacy and embed reliable and compliant reprocessing systems to ensure adequate cleaning, disinfection and sterilisation (as required), as well as the ability to identify patients who have encountered reusable equipment in the event of an infection outbreak (as necessary).

Monitoring and Evidence Examples

- The pharmacy's policies and procedures covering respiratory hygiene, physical distancing, standard and transmission precautions, and outbreak response.
- Records of updates to policies reflecting changes in guidelines or regulations.
- Demonstrated evidence that appropriate cleaning and hygiene products are available for pharmacy staff to use (e.g. evidence-based cleaning products, face masks, tissues, hand sanitiser).
- The pharmacy's hand hygiene protocols and visible hand hygiene stations with signage encouraging use by staff and consumers.
- Materials promoting respiratory hygiene, cough etiquette and physical distancing.

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- Records of completed workshops or training related to infection control and prevention, hand hygiene processes, respiratory hygiene, cough etiquette, physical distancing and effective cleaning and maintenance protocols for routine and outbreak situations.
- Educational materials distributed to staff and consumers about infection control.
- The pharmacy's aseptic technique protocols and any associated checklists, audit records or compliance documentation verifying correct aseptic technique.
- The pharmacy's documented processes for the insertion of invasive medical devices (if applicable) and records of completed training for pharmacy staff involved in such activities.
- The pharmacy's routine cleaning schedule aligned with infection control guidelines.
- Examples of emergency plans detailing pharmacy operations during pandemics or outbreaks and how the pharmacy will collaborate with local health authorities for coordinated outbreak response. This can be incorporated as part of the pharmacy's business continuity plan, or a separate document.
- Evidence that the cleaning products used by the pharmacy are listed on the Australian Register of Therapeutic Goods.
- Identified infection risks recorded in the pharmacy's risk management plan and strategies to manage such risks.
- The pharmacy's policy for staff vaccination against preventable diseases including recommended vaccinations for pharmacy staff.
- The pharmacy's risk-based processes for managing and preventing acute infections among pharmacy staff.
- The pharmacy's protocols for cleaning reusable equipment, instruments and devices and records of the use of such equipment with patients (as necessary).

Resources and Information

Australia Commission on Safety and Quality in Healthcare

- Australian Guidelines for the Prevention and Control of Infection in Healthcare (2019).
- Infection prevention and control resources for consumers.
- Principles for aseptic technique: Information for healthcare workers.
- Environmental cleaning and infection prevention and control resources.
- The National Hand Hygiene Initiative (NHHI)
- NSQHS Standards Workforce Immunisation Risk Matrix
- PCHS24/01 Advisory: National Safety and Quality Primary and Community Healthcare Standards requirements for reprocessing of reusable medical devices in practices Advisory

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2.7 Communicating for Safety

Intention

All relevant stakeholders, including patients and healthcare providers, receive timely information and alerts to support safe and seamless coordination of care.

Outcome

I am empowered to actively engage in my care through communication processes that ensure safer, more transparent, and consistently high-quality healthcare.

Standard Subclause

The pharmacy shall adopt timely, purpose-driven and effective communication strategies that support continuous, coordinated and safe care for patients.

2.3.3.1 The pharmacy shall have processes that use at least three patient identifiers to ensure patients are correctly identified.

Note:

Correctly identifying patients is particularly important when handing out prescription medications or dose administration aids.

2.3.3.2 The pharmacy shall have processes to:

a. Correctly match patients to their care

b. Ensure essential information is documented in a patient's healthcare record.

2.3.3.3 The pharmacy shall support its pharmacists to refer patients to other services and collaborate with other care providers by:

a. Using structured communication processes.

b. Considering the patient's risks, goals and preferences for health care.

c. Communicating information that is current, comprehensive and accurate.

2.3.3.4 The pharmacy shall use its communication processes to effectively communicate critical information, alerts and risks in a timely way when they emerge or change to:

a. Relevant healthcare providers involved in the patient's care.

b. Patients, carers and families, in accordance with the patient's preferences.

2.3.3.5 The pharmacy shall have communication processes for patients and consumers to directly communicate critical information and risks about care to their healthcare providers.

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Reflective Questions

- How effectively are our communication processes ensuring patient safety and seamless care coordination?
- What steps have we taken to correctly identify patients using multiple identifiers, and how consistent are we in their application?
- In what ways do our documentation practices support accurate and comprehensive recording of patient healthcare information?
- How do we ensure timely communication of critical information, alerts, and risks to all relevant stakeholders, including patients and healthcare providers?
- Are our referral processes and collaborations with other healthcare providers meeting the needs and preferences of our patients?
- How do our policies allow patients to directly communicate their concerns and risks about their care to healthcare providers?
- What mechanisms have we put in place to regularly review and update communication protocols to adapt to emerging risks or changes?
- How do we foster transparency and include patient, carer, and family perspectives when critical information needs to be conveyed?

Key Tasks

- Establish and follow a robust process using at least three identifiers (e.g., name, date of birth, and address) to ensure patients are correctly identified and matched with their medicines or care.
- Accurately and comprehensively document patient healthcare information in their healthcare record and provide access to patient healthcare records to those involved in their care.
- Establish systems, including structured communication protocols and tools (e.g. referral forms, medication lists), to support referral to other healthcare providers, when appropriate. Consider patient risks, goals, and preferences in all collaborative healthcare activities.
- Set up communication channels for patients and consumers to directly share concerns and/or critical information with pharmacists in the pharmacy. This may include using feedback to improve communication protocols and care delivery practices.
- Periodically review communication protocols to adapt to emerging risks or changes, including patient, caregiver, and family perspectives in reviews to foster transparency.

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Monitoring and Evidence Examples

- The pharmacy has a procedure for the use of multiple identifiers, such as name, date of birth, and address to correctly match patients to their medicines and care.
- Patient healthcare records demonstrating comprehensive and accurate documentation of healthcare information.
- Examples of referral forms or templates used to communicate patient information to other healthcare providers.
- Records of correspondence, collaborative care meetings or calls showcasing coordinated efforts between providers.
- Feedback from patients regarding the effectiveness of referral processes, and evidence to demonstrate feedback has been incorporated to improve.
- Logs of alerts or notifications sent to patients or healthcare providers about critical risks or information.
- Protocols for communicating urgent information promptly.
- Patient satisfaction surveys regarding the timeliness and clarity of communication.
- Documentation of feedback channels, such as suggestion boxes, online forms, or direct communication logs.
- Reports on how feedback has been integrated into improving communication and operational protocols.
- Records of patient concerns being addressed and resolved effectively.
- Minutes from meetings indicating the reviewing and updating of communication protocols based on emerging risks.
- Updated versions of policies and procedures reflecting changes made from reviews.
- Demonstrated use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures.

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2.8 Recognising and Responding to Serious Deterioration and Minimising Harm

Intention

Patient safety is prioritised through timely care escalation when a patient experiences a health crisis beyond a pharmacist's scope of practice.

Outcome

I trust that my community pharmacy will quickly recognise and respond appropriately to any acute health issue I may experience whilst in the pharmacy.

Standard Subclause

The pharmacy shall have systems in place to recognise and respond to acute deterioration in patients and escalate care appropriately.

2.3.4.1 Pharmacists shall use the pharmacy's processes to

a. Recognise deterioration in a patient's physical, mental or cognitive health.

b. Respond to a patient within their scope of practice and call for emergency assistance.

c. Notify a patient's other relevant healthcare providers, carers or family when their health care is escalated.

2.3.4.2 The pharmacy shall have processes to:

a. Have processes to respond to patients who are distressed, have expressed thoughts of self-harm or suicide, or have self-harmed.

b. Have processes to respond to patients who present a risk of harm to others.

c. Provide information on accessing other services to patients with healthcare needs beyond the scope of the pharmacy.

d. Have a process that supports crisis intervention that is aligned to legislation.

Reflective Questions

- How effectively do we recognise signs of acute physical, mental, or cognitive health deterioration in our patients?
- Do our current processes for responding to patient distress align with legislative requirements and best practice standards?
- Are our procedures for escalating care clear, accessible, and adhered to during emergencies?
- How well do we coordinate with other healthcare providers, carers, or family members when a patient's care is escalated?
- Do we have protocols in place for responding to patients who express thoughts of self-harm or pose a risk to others?
- Are we providing adequate information and resources to patients whose healthcare needs extend beyond our scope of practice?
- Do we review and update our crisis intervention processes regularly to ensure they remain compliant with current legislation?
- How do we document evidence of compliance with this standard, and is it sufficient to demonstrate our commitment to patient safety?

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Key Tasks

- Develop processes to support pharmacy staff to:
  - **Recognise deterioration in a patient's physical, mental or cognitive health.**
  - **Respond to a patient who is distressed, has expressed thoughts of self-harm or suicide, or has self-harmed.**
  - **Respond to a patient who presents a risk of harm to others.**
  - **Respond to a patient within their scope of practice and call for prompt emergency assistance, if required.**
  - **Notify a patient's other relevant healthcare providers, carers or family when health care is escalated.**
- Facilitate training for pharmacy staff so they have the necessary skills and equipment to recognise and respond to serious deterioration, distress or harm, within their scope of clinical practice.
- Source, and make available, information on accessing external mental health services (e.g. general practitioners, community mental health services or hospital emergency departments).
- Facilitate access for pharmacy staff to post-incident support services (e.g. trauma counselling, employee assistance programs), as needed.

Monitoring and Evidence Examples

- The pharmacy's processes for recognising and responding to serious deterioration, distress or harm, and escalating care.
- Staff training records which reflect staff education and training in the areas of responding to serious deterioration, distress or harm, escalating care, and accessing emergency assistance.
- Easily accessible and available information for staff on external mental health services.
- Posters and other resources such as flow charts that are accessible and displayed in appropriate locations for staff to refer to in the event of an emergency.
- Demonstrated evidence that pharmacy staff have access to trauma counselling and/or an employee assistance program if they require support.

Resources and Information

Australian Commission on Safety and Quality in Health Care

- NSQHS Standards User Guide for health services providing care for people with mental health issues
- NSQHS Standards User Guide for health service organisations providing care for patients with cognitive impairment or at risk of delirium

Australian Government Department of Health, Disability and Ageing

- Mental health and suicide prevention resources

National Institute for Health and Care Excellence

- Violence and aggression: short-term management in mental health, health and community settings.

Other resources:

- Mental Health First Aid International
- Life in Mind: Suicide Prevention

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Domain 3.

Partnering with Consumers and the Community

Pharmacies play a role in promoting health and wellbeing, not only through safe and effective medication management but also by fostering meaningful partnerships with consumers and the broader community. This Domain outlines the tasks that support inclusive, respectful, and culturally responsive engagement, ensuring that consumer voices are actively heard and valued in the design, delivery, and evaluation of pharmacy services.

The Partnering with Consumers and the Community Domain covers the following areas:

3.1 **Partnering with Patients in their own Care**

3.2 **Health Literacy**

3.3 **Partnering with Consumers in Pharmacy Design and Governance**

3.4 **Comprehensive Care**

By building trust and collaboration, pharmacies can enhance safety, improve quality of care, and contribute to more equitable health outcomes.



3.1 Partnering with Patients in their own Care

Intention

Health outcomes are enhanced through engagement with patients and consumers to meet individual needs.

Outcome

I am a partner in my own healthcare, and I make informed decisions about my care in collaboration with my community pharmacist and pharmacy assistants.

Standard Subclause

The pharmacy shall develop, implement and maintain systems to partner with patients in their own health care.

1.1.5.1 The pharmacy shall:

a. Use a Charter of Rights consistent with the Australian Charter of Healthcare Rights.

b. Have processes to support pharmacy staff to apply the principles of the Charter of Rights in the planning and delivery of health care.

c. Make the Charter of Rights easily accessible for patients and consumers.

d. Ensure its informed consent processes comply with legislation, Codes of Conduct and the needs of the community.



Standard Subclause

1.1.5.2 The pharmacy shall have processes to identify:

a. The capacity of a patient to make decisions about their own care.

b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves.

Note:

The capacity of a patient to make decisions for themselves can be influenced by their age, language, mental health conditions, disability or other health conditions. Pharmacists should be aware of relevant jurisdictional legislation regarding the age of consent for medical care and should take reasonable steps to determine if a patient's decision-maker has the authority to give consent and make decisions on their behalf. Details of substitute decisionmakers should be recorded in the patient's healthcare record.

1.1.5.3 The pharmacy shall have processes for pharmacists to partner with patients and/or their substitute decision-maker to plan, effectively communicate, set and review goals and make decisions about their current and future care.

1.1.5.4 The pharmacy shall support pharmacy staff to form partnerships with patients and consumers so that they can be actively involved in their own health care.

Reflective Questions

- How do we actively understand and engage with the Charter of Rights in our interactions with patients?
- In what ways do we involve patients in the planning and decision-making processes about their care?
- Are our processes for informed consent clear, compliant with legislation, and effectively communicated to patients?
- How do we ascertain a patient's capacity to make decisions regarding their healthcare, and do we have a protocol for engaging with substitute decision-makers when necessary?
- What methods do we use to set and review goals with patients, ensuring they are actively engaged in their health care?
- What training and support mechanisms are in place to help pharmacy staff form partnerships with patients and consumers?

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Key Tasks

- Promote the Charter of Rights by displaying the Charter prominently to ensure patients and consumers have easy access and are made aware of their rights when receiving healthcare.
- Support pharmacy staff to incorporate the principles of the Charter into their daily practice and interactions with patients.
- Support pharmacy staff to engage in shared decision-making by involving patients in the planning and decisions around their care.
- Establish and follow informed consent processes which are clear and well communicated to patients. This includes explaining the risks and benefits of treatments, ensuring the patient's understanding, and getting written consent when necessary.
- Implement a system to identify if a patient's capacity to make decisions about their own care is compromised. Consideration needs to be given to factors like age, language barriers, mental health, disabilities, or other health conditions. Referral to the patient's GP for assessment should be facilitated as required.
- If the pharmacy is notified that a substitute decision-maker has been appointed for a patient, pharmacy staff should work with substitute decision-makers and record their authority in the patient's healthcare record.
- Facilitate access to training and tools for pharmacy staff to enable effective partnerships with patients and consumers. Education and training may include communication skills, cultural competency training, and education on relevant laws and ethical guidelines.

Monitoring and Evidence Examples

- The Charter of Rights is displayed prominently in the pharmacy on posters, and/or provided in pamphlets for patients to take home.
- Survey responses (e.g. Patient Reported Experience Measures (PREMs) via the use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures) or feedback forms, where patients have indicated their satisfaction and active involvement in their care decisions.
- Documentation of patient consultations which demonstrates engagement in shared decision-making through a discussion of treatment options, goal setting, and patient preferences.
- The pharmacy has a documented informed consent procedure which is understood and followed by staff.
- Template forms are available for capturing informed consent.
- Evidence of signed consent forms, or records of verbal consent, for treatments and procedures.
- Examples of written information provided to patients detailing the risks and benefits of specific treatments in easy-to-understand language.
- The pharmacy has a procedure for referring patients for formal assessment if there are concerns about their ability to make decisions about their own care.
- Documented evidence of substitute decision-makers (if applicable), including their contact details and authority documented in the patient's healthcare record.
- Records of training for pharmacy staff related to communication skills, Charter of Rights, cultural competency training, and education on relevant laws and ethical guidelines.

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Resources and Information

Australian Commission on Safety and Quality in Health Care

- Australian Charter of Healthcare Rights
- Informed consent

Pharmaceutical Society of Australia

- Code of Ethics for Pharmacists

Australian Government Office of the Australian Information Commissioner

- Australian Privacy Principles

Other useful resources

- Capacity Australia



3.2 Health Literacy

Intention
Information and communication styles are tailored based on the needs of their patients.

Outcome
I am given the information I need, in a way I can understand, to help me make decisions about my care.

Standard Subclause

Pharmacy staff shall communicate with consumers in a way that supports effective partnerships.

1.1.6.1 Pharmacy staff shall communicate with patients and consumers about health and health care in a way that:

a. Is tailored to the patient's needs and preferences.
b. Is easily understood.
c. Addresses the need for ongoing health care.

1.1.6.2 The pharmacy shall make information available to consumers on:

a. The services available.
b. Opening hours and how to access healthcare.
c. Who can access the services.
d. Estimated costs of service delivery.
e. Alternative service when the pharmacy is unavailable.
f. Service location(s) and access details if relevant.
g. Mechanism for providing feedback and contact details for the appropriate healthcare complaints authority.

Reflective Questions

- How are we tailoring the information we provide to our patients and consumers to meet their specific needs and preferences?
- Do our patients find the information we provide easy to understand? How do we assess this?
- What processes do we have in place to support consistently addressing ongoing health care requirements for our patients?
- How are the services we offer clearly communicated? Does it include access details and estimated costs?
- Where do we advertise or communicate alternative services that are available to patients when our pharmacy is closed?
- Is there a transparent feedback mechanism in place for patients to share their experiences and concerns?
- How do we foster effective partnerships with our patients and consumers through clear and continuous communication?

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Key Tasks

- Establish processes that promote clear communication with patients and consumers, both written and verbal, in a way that is open, clear and supportive.
- The pharmacy has access to a variety of contemporary, evidence-based information sources, which pharmacy staff use to provide to patients and consumers to support their healthcare needs and understanding.
- Provide training and education to pharmacy staff on health literacy and the different forms of information and communication they can use to meet the specific needs and preferences of patients and consumers.
- Implement method(s) for providing clear information on the services available within the pharmacy, including how and when services can be accessed, estimated costs and alternative services.
- Implement transparent feedback mechanisms (e.g. digital surveys, suggestion boxes, QR codes) that enable patients and consumers to share their experiences and concerns.

Monitoring and Evidence Examples

- Examples of sources of contemporary, evidence-based information sources which pharmacy staff can access to share with patients.
- Alternative tailored information sources and services which can be accessed to address specific needs and preferences of patients and consumers.
- The pharmacy has a procedure or protocol related to the effective provision of information to support patients and consumers.
- Records of training or education provided to pharmacy staff related to health literacy and/or tailoring communication and information to the specific needs of the patient and/or consumer.
- Evidence of feedback from patients indicating that information provided by the pharmacy was easy to understand.
- Demonstrated evidence of pharmacy service information that is provided to patients and consumers.
- Evidence the pharmacy has transparent feedback mechanisms which enable patients to share their experiences and concerns. (e.g. demonstrated use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures).

Resources and Information

Australian Commission on Safety and Quality in Health Care

- Supportive resources on health literacy

Therapeutic Goods Administration

- Consumer Medicines Information

Other useful resources

- Better Health Channel
- Translating and Interpreting Service
- Healthdirect

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3.3 Partnering with Consumers in Pharmacy Design and Governance

Intention

Pharmacy services are improved through feedback from patients and communities, and monitoring of their experiences.

Outcome

My opinion is heard and considered in the design, development, delivery and review of services provided by my community pharmacy.

Standard Subclause

Consumers shall be partners in the planning, design, monitoring and evaluation of pharmacy services.

1.1.7.1 The pharmacy shall have a systematic approach to endeavouring to partner with patients, consumers and communities to seek and incorporate their views and experiences into the planning, design, monitoring and evaluation of services.

Note:

Co-design and co-development between health professionals, individuals and communities has the potential to build trust, empower individuals and communities and improve health outcomes. Pharmacists should invest in meaningful engagement with patients, carers and communities to design services that have the greatest impact for all.

Reflective Questions

- How does the pharmacy actively engage with patients, consumers, and the community in the planning and design of services?
- What processes are in place to ensure that patient feedback is incorporated into the evaluation and improvement of services?
- How does the physical environment of the pharmacy support safe and effective care?
- How are the cultural needs and considerations of the community addressed in the pharmacy's design and operation?
- What measures are taken to create a patient-directed environment that enhances safety, privacy, and cultural sensitivity?

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Key Tasks

- Conduct regular patient and consumer satisfaction surveys **(e.g. patient-reported experience or outcome measures)** to gather feedback on pharmacy services.
- Collaborate with community leaders or diverse community members to understand and address specific needs and preferences.
- Engage with patients and consumers to gather their feedback on pharmacy service delivery. This could be completed through ad hoc discussions or focus group sessions coordinated by individual pharmacies or pharmacy group head offices as appropriate
- Utilise feedback in the design and layout of the physical pharmacy space to enhance comfort and accessibility (when feasible).
- Incorporate feedback in the design and delivery of pharmacy services when feasible.

Monitoring and Evidence Examples

- Records of regular patient and consumer feedback on pharmacy services.
- **Patient-reported experience or outcome responses via the use of an approved (conformant) system for the collection, management and submission of Patient Recorded Experience Measures and Patient Recorded Outcome Measures.**
- Documentation of discussions or focus group meetings.
- Feedback from community representatives on the effectiveness of cultural considerations within pharmacy operations.
- Examples of consultation and subsequent modifications to pharmacy design or services to address the specific needs of the community.
- Policy documents outlining the pharmacy's commitment to patient-directed care.

Resources and Information

New South Wales Government Agency for Clinical Innovation

- Co-Design Toolkit

Patient Experience Agency

- Fundamentals of Co-design in Healthcare

Australian Healthcare and Hospitals Association

- Experience-based Co-design Toolkit

Australia Commission on Safety and Quality in Health Care

- Implementing Patient Experience / Patient-Reported Experience Measure (PREMs)
- Patient Reported Outcome Measures

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3.4 Comprehensive Care

Intention

The pharmacy delivers coordinated care that is clinically appropriate and takes into consideration the patient's preferences.

Outcome

I receive safe, high quality health care that is tailored to meet my needs and preferences.

Standard Subclause

The pharmacy shall deliver coordinated care that is clinically appropriate and takes into consideration the patient's preferences. This care shall be planned, delivered in collaboration with the patient, and consider the effect of the patient's health issues on their life and wellbeing. Comprehensive care may be a discrete episode of care or part of an ongoing comprehensive care plan and be within the context of clinical services provided by the pharmacy.

2.3.2.1 The pharmacy shall have a system to:

a. Collaborate with other healthcare providers involved in a patient's care.

b. Support collaboration with other care providers to develop a coordinated approach to the planning and delivery of health care.

c. Facilitate reporting to a patient's other relevant care providers.

d. Record collaboration, or attempts to collaborate, with other healthcare providers.

2.3.2.2 The pharmacy shall have processes to support health education and promotion, illness prevention and early intervention for patients, considering its patient population.

2.3.2.3 The pharmacy shall have processes to:

a. Check if a patient is of Aboriginal and/or Torres Strait Islander origin for services or settings where this is relevant or appropriate (e.g., immunisations).

b. Record this information in a relevant part of the patient's healthcare record (e.g., Australian Immunisation Register).

c. Use this information to optimise the planning and delivery of healthcare.

2.3.2.4 The pharmacy shall support its staff to meet the individual needs of its patients, including those:

a. With a disability.

b. From diverse populations.

c. Who are receiving palliative or end of life care.

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Reflective Questions

- How effectively are we collaborating with other healthcare providers involved in our patients' care?
- Do we have systems in place to support coordinated care planning and delivery?
- Are we actively facilitating communication and reporting to our patients' other relevant care providers?
- Do our records reflect collaboration efforts with other healthcare providers?
- How comprehensive are our health education and promotion initiatives for our patient population?
- Are we consistently identifying and recording patients of Aboriginal and/or Torres Strait Islander origin when relevant and appropriate?
- How well are we using this information to optimise healthcare planning and delivery?
- Have we developed processes to meet the individual needs of patients with disabilities, from diverse populations, and those receiving palliative or end-of-life care?
- Are we supporting our staff to effectively address the unique needs of our patients?

Key Tasks

- Establish, maintain and follow processes for:
  - **Identifying and documenting other healthcare providers involved in a patient's care**
  - **Timely referral and/or exchange of patient information between healthcare providers**
  - **Recording collaboration, or attempts to collaborate, with other healthcare providers**
- Identify health education and promotion priorities relevant to the pharmacy's patient population and community.
- Organise workshops or educational campaigns aimed at improving health literacy among patients. Provide current and relevant materials such as brochures and flyers tailored to patient needs.
- If appropriate, conduct evidence-based screening activities to aid early intervention. Refer patients to other relevant healthcare providers as required.
- Incorporate prompts into pharmacy services to discuss preventative health and health promotion with patients, carers and consumers.
- Utilise social media, websites, and mobile applications to engage with patients and share relevant preventative health information and updates.
- Embed a process for identifying patients of Aboriginal and/or Torres Strait Islander origin for services where this information is relevant, incorporating self-identification processes.
- Update healthcare records to include cultural background and relevant demographic data, ensuring this information is utilised to optimise healthcare delivery.
- Provide continuous training to pharmacy staff to sensitively and effectively identify patients of Aboriginal and/or Torres Strait Islander origin. Highlight the importance of culturally appropriate care.
- Provide education and/or resources for pharmacy staff to improve their awareness and understanding of patients who are receiving palliative or end of life care, and how to support them in a sensitive manner.

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Monitoring and Evidence Examples

- The pharmacy's procedure for identifying other healthcare providers, exchanging patient information, referring patients as required, and recording collaboration, or attempts to collaborate, with other healthcare providers.
- Records of collaboration or attempts to collaborate with other healthcare providers involved in the patient's care.
- Demonstrated evidence of education campaigns conducted by the pharmacy, including examples of brochures, flyers, and digital content provided to patients.
- Analytics and engagement metrics from digital platforms, including social media, websites, and mobile apps used to share health tips, news, and updates.
- Demonstrated process and evidence of how the pharmacy records cultural background information, such as Aboriginal and/or Torres Strait Islander status, in relevant parts of the healthcare record. This may include forms on which patients can identify as being of Aboriginal and/or Torres Strait Islander origin.
- Records of training related to identifying and supporting patients receiving palliative or end of life care.

Resources and Information

Health and Cultural Safety Strategy

- Aboriginal and Torres Strait Islander Health and Cultural Safety Strategy 2020–2025

Pharmaceutical Society of Australia

- Guidelines for pharmacists supporting Aboriginal and Torres Strait Islander peoples with Medicines Management.

Australian Commission on Safety and Quality in Health Care

- The National Consensus Statement: Essential elements for safe and high-quality end-of-life care

Australian Government Department of Health

- About preventive health in Australia



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Domain 4.

Business Governance and Operations

Domain 4 Business Governance and Operations guides pharmacy owners, managers, and staff in implementing sound business governance practices within Australian community pharmacies, ensuring professional and statutory obligations are met, workload and risks are managed, and continuity of service is maintained for patients, consumers and the broader community.

The sections within the Business Governance and Operations Domain are:

4.1 Legal and Professional Obligations

4.2 Business Viability

4.3 Business Continuity Plan

4.4 Pharmacy Premises

4.5 Safety and Security

4.6 Information Technology (IT)

4.7 Advertising of Pharmacy Services and Therapeutic Goods

4.8 Stock Management

4.9 Human Resources (HR)

Having robust systems and processes in place in these areas supports pharmacy staff to fulfil their employment responsibilities in a safe manner and facilitates the effective delivery of medicines and pharmacy services to local communities.

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4.1 Legal and Professional Obligations

Intention

Compliance with all relevant legislation, regulations, codes, standards and professional obligations is achieved through robust systems and processes.

Outcome

I know that I am receiving care from a lawful and reputable pharmacy.

Standard Subclause

The pharmacy shall have systems to ensure compliance with all statutory and professional obligations including State and Territory legislation.

1.2.1.1 The pharmacy shall support the provision of the medicines and services it offers by complying with legislative and professional requirements for operating a pharmacy.

1.2.1.2 The pharmacy shall have a systematic approach to ensure each owner and pharmacist employed or contracted by the pharmacy is registered with the Pharmacy Board of Australia.

1.2.1.3 The pharmacy shall ensure all healthcare providers employed or contracted by the pharmacy maintain profession-specific registration as appropriate to their profession.

1.2.1.4 The pharmacy shall maintain appropriate professional indemnity, public liability, and workers' compensation insurance.

1.2.1.5 As an employer, the pharmacy shall comply with its legal responsibilities for employing staff including, but not limited to:

a. Employee payroll tax.
b. Pay rates and conditions.
c. Superannuation payments.
d. Keeping staff records.
e. Workplace health and safety.
f. Leave entitlements.

1.2.1.6 Pharmacy proprietors shall maintain oversight of how their pharmacies are operating.

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Reflective Questions

- In what ways does the pharmacy ensure that it has adequate professional indemnity, public liability, and workers' compensation insurance in place?
- How do we ensure compliance with our legal responsibilities, including payroll tax, pay rates, superannuation payments, and workplace health and safety requirements?
- How effectively are staff records kept, including their leave entitlements and employment conditions?
- What strategies do we have in place to ensure oversight of pharmacy operations and compliance with legal and regulatory obligations, such as the Corporations Act 2001 and the Fair Work Act 2009?
- What evidence can we provide to demonstrate our compliance with our legal and professional obligations?



Key Tasks

- Implement a mechanism to identify, understand and comply with all legal (Commonwealth, State and Territory) and professional requirements applicable to community pharmacy operations. The mechanism should include a process of monitoring changes in legislation and professional requirements and how the pharmacy responds.
- Establish and manage a system to monitor the ongoing AHPRA, Pharmacy Board of Australia (PBA) registration status of pharmacy owners and pharmacists working within the pharmacy (noting that individuals make a declaration of compliance with the Professional Indemnity and Continuing Professional Development requirements as part of the health practitioner registration process).
- Where healthcare providers (e.g. nurses, naturopaths, diabetes educators) are employed by the pharmacy, implement a system to ensure they hold current, profession-specific registration as appropriate to their profession.
- Maintain current and adequate professional indemnity, public liability, and workers' compensation insurance.
- Implement a mechanism to identify, understand and comply with all legal (Commonwealth, State and Territory) responsibilities of being an employer, including:
  - Accurate and timely payment of payroll taxes.
  - Adherence to legal pay rates and employment conditions as per the Pharmacy Industry Award.
  - Consistent and correct superannuation contributions for all staff.
  - Thorough records of staff conditions and leave entitlements.
  - Practices that uphold workplace health and safety obligations.

  The mechanism should include a process for monitoring changes in legislation and how the pharmacy responds.
- Incorporate compliance requirements into policy and procedural documentation, as appropriate.
- Pharmacy owners oversee and input into the operation of the pharmacy.

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Monitoring and Evidence Examples

- Demonstrated evidence of how the pharmacy monitors changes in legislation and professional requirements and ensures compliance.
- Up-to-date records verifying the registration of all pharmacy owners and pharmacists with the Pharmacy Board of Australia.
- Up-to-date records verifying the profession-specific registration of healthcare providers employed by the pharmacy (e.g. nurses, naturopaths, diabetes educators).
- Records of professional indemnity, public liability, and workers' compensation insurance policies, including certificates of currency and renewal notices.
- Records of payroll tax and superannuation contributions such as payslips, bank transaction records, clearing house records or statements from payroll servicing companies.
- Comprehensive records of staff employment conditions (e.g. employment contracts) and tracking of leave entitlements given to pharmacy staff.
- The pharmacy's written policies and procedures related to the legal and professional obligations of operating a pharmacy and employing staff.
- A signed annual attestation statement by the pharmacy owner(s) acknowledging their accountability for overall legal compliance in the pharmacy. This statement can be combined with the safety and quality attestation statement (See Clinical Governance and Leadership Domain), or a separate document.

Resources and Information

Pharmacy Board of Australia

- Guidelines for Proprietor Pharmacists
- Look up a Practitioner

Pharmaceutical Society of Australia

- State and Territory Legislation links

Australian Commission on Safety and Quality in Healthcare

- Fact Sheet 7: Governing Body Attestation Statement

Healthcare Professional Registration Schemes

- AHPRA National Boards
- Naturopaths and Herbalists Association of Australia
- Australian Diabetes Educators Association
- Nutrition Society of Australia
- Speech Pathology Australia

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4.2 Business Viability

Intention

Effective business operations maintain the profitability, viability and sustainability of the community pharmacy.

Outcome

My community pharmacy will be there for me in the future.

Standard Subclause

The pharmacy shall have business plans and strategies in place to ensure the pharmacy operates effectively and maintains profitability, viability and sustainability.

1.2.2.1 The pharmacy shall define and document its ownership structure appropriate for the size and type of pharmacy, including:

a. The details of all pharmacy owners, business partnerships, shareholder or unit holders' agreements (as applicable).

b. The details of the pharmacy model and any corporate, banner or not-for-profit entities associated with the pharmacy (as applicable).

c. Reporting requirements to inform all owners of the performance of, and any risks to, the pharmacy business.

1.2.2.2 The pharmacy shall undertake comprehensive business planning, which should be documented and include the following as appropriate for the size and type of pharmacy:

a. A business plan including the dispensary, retail and pharmacy services offered.

b. The resources (e.g., funds, finance, staff, stock, technology and equipment) required to execute the business plan.

c. A budget and forecast for the year ahead (at a minimum).

Note:

Business planning should take into consideration consumer demographics, patient needs, the pharmacy model and structure, and financial and market pressures.

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Standard Subclause

1.2.2.3 At least annually, the following shall be analysed and reported to all pharmacy owners:

a. Total business performance, including any issues identified from its analysis.
b. Financial and business trends.
c. Future financial commitments and budget forecasting.
d. Risks to the viability of the pharmacy business.

Note:

Other items may also be reported in addition to the above, as determined by the owners and managers of the pharmacy.

1.2.2.4 The pharmacy shall implement strategies to address any issues identified during the analysis of its total business performance to maintain viability.

1.2.2.5 The pharmacy shall consider sustainable business practices that support the business model.

Reflective Questions

- How do we document the details of all pharmacy owners and any business partnerships or shareholder agreements?
- How effective are the reporting requirements in informing all owners about the performance and risks of the pharmacy business?
- Do we have a comprehensive business plan that includes dispensary, retail and pharmacy services offered?
- How have we identified the necessary resources such as funds, finance, staff, stock, technology, and equipment required to execute the business plan?
- How are consumer demographics and patient-needs adequately considered within our business planning?
- How regularly do we analyse and report the total business performance to all pharmacy owners?
- Do we accurately identify and address issues from our performance analysis, including tracking financial and business trends?
- What risks to our pharmacy business have we identified, and how are we addressing them?
- Have we considered environmental, social, and economic sustainability in our operations? If so, how are they being incorporated to ensure sustainable business practices?

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Key Tasks

- Maintain records and documentation of pharmacy ownership including pharmacy owners, business partnerships and shareholder agreements, if any.
- Develop a business plan, appropriate for the size and complexity of the pharmacy. The plan should contain evidence of:
  - **The dispensary, retail, and pharmacy service offerings.**
  - **Required resources such as staff, stock, technology, and equipment.**
  - **Budgets and forecasts.**
  - **This may include community consultation and consideration of consumer feedback to guide future direction.**
- Create and maintain a risk management plan for the pharmacy to identify, assess, and mitigate potential risks, including financial, operational, and reputational risks.
- Establish and maintain a system to:
  - **Regularly review and analyse the pharmacy's financial performance and business risks.**
  - **Report financial performance and business risks to all pharmacy owners at least annually.**
  - **Promptly escalate information regarding risks to the pharmacy business, as required.**
  - **Continuously implement strategies to maintain the viability of the pharmacy.**
- Embed environmentally friendly and sustainable business practices where appropriate (e.g. recycling delivery packaging, reducing paper waste, eliminating the use of plastic bags, electricity saving initiatives).

Monitoring and Evidence Examples

- Records of pharmacy ownership.
- A documented business plan that is fit for purpose and may include business strategy and operational objectives, considers business performance and measurement of business metrics.
- Documentation and/or evidence to demonstrate effective and regular business planning.
- Documented evidence such as resource allocation records, performance and financial reports, and/or records of regular business plan updates to demonstrate the pharmacy's business plan is used effectively and maintained to support business needs.
- The pharmacy's actively managed risk management plan that identifies and assesses potential risks and details the strategies implemented to address these risks.
- Demonstrated evidence of the pharmacy's system for analysing business performance and risks, reporting performance and risks to all pharmacy owners, escalating business risks as required, and implementing strategies to maintain viability.
- Reports from pharmacy management software used to track key metrics (e.g. prescription numbers, number of services delivered, stockturn) and support the analysis of the pharmacy's business performance.
- Demonstrated evidence of environmentally sustainable practices appropriate for the size, location and complexity of the pharmacy.
- Evidence of improved efficiency resulting from sustainable practices (e.g. a reduction in electricity usage).

Resources and Information

Australian Government Business

- Support for Businesses in Australia

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4.3 Business Continuity Plan

Intention

Business continuity planning enables community pharmacies to effectively respond to unexpected emergencies like natural disasters, pandemics, building fires, or the unplanned absence of key pharmacy staff, so that the community is not left without essential services.

Outcome

In an emergency my community pharmacy has a plan to ensure my essential health needs continue to be met.

Standard Subclause

The pharmacy shall develop and maintain a business continuity plan for unexpected emergencies such as natural disasters, pandemics, building fires or unplanned absences of key pharmacy staff.

1.2.3.1 The pharmacy shall have a documented business continuity plan which includes strategies and actions to limit the impact to the operations of the pharmacy in an emergency.

1.2.3.2 All pharmacy staff shall be trained in the pharmacy's business continuity plan and understand their role and how to respond in the case of an emergency.

1.2.3.3 The business continuity plan shall be reviewed at least annually, and strategies and actions updated as required.

Reflective Questions

- How well does our pharmacy's business continuity plan address potential emergencies specific to our region, such as natural disasters, pandemics, or fires?
- What strategies do we have in place to ensure all staff members are adequately trained and aware of their roles during an emergency? What additional training might be necessary?
- How often do we review and update our business continuity plan to ensure it remains relevant and effective?
- How can we better utilise professional organisations, banner groups, wholesalers and partnerships with local healthcare providers to support our operations in the event of an emergency?
- What communication strategies could be implemented to ensure patients remain informed about service changes during an emergency?
- Are there any gaps in our current plan that could affect our ability to maintain uninterrupted service delivery?

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Key Tasks

- Develop a documented business continuity plan to be activated in the event of an emergency. The plan should include strategies and actions for maintaining continuity of critical pharmacy services (e.g. supply of medicines) and patient care.
- Educate all pharmacy staff on the business continuity plan, when it would be activated and their roles and responsibilities in the event of an emergency.
- Review the pharmacy's business continuity plan at least annually to verify that it remains fit-for-purpose, and amend the plan as required.
- Develop and implement an effective communication plan to keep patients and consumers informed about pharmacy service changes and availability during emergencies.

Monitoring and Evidence Examples

- The pharmacy's business continuity plan, which includes strategies and actions to mitigate the impact of various emergencies.
- Minutes of staff meetings or training records related to the business continuity plan, including attendance lists and any training materials.
- Evidence to reflect an annual review of the business continuity plan, highlighting updates and improvements made in response to environmental changes, new threats or best practice.
- Evidence of communications implemented during an emergency, including emails, social media posts, and posters or community notices keeping the community informed.

Resources and Information

New South Wales Government, Small Business Commissioner

- How to build a Business Continuity Plan

Queensland Government, Business Queensland

- Business Continuity Planning

Government of South Australia, Human Services

- Toolbox including Business Continuity Example

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4.4 Pharmacy Premises

Intention

The pharmacy has appropriately maintained premises and facilities to support the safe delivery of products and services.

Outcome

I receive care in a well maintained and safe environment.

Standard Subclause

The pharmacy shall have systems to ensure the pharmacy premises are appropriately maintained.

1.2.4.1 The pharmacy shall:

a. Ensure the pharmacy premises' lease is compliant with the business, and that the business complies with the lease's requirements.

b. If the pharmacy premises are owned, ensure the property is maintained at a standard that is appropriate to allow the safe and professional conduct of the pharmacy business.

c. Not enter, or be bound by, any contracts (including tenancy leases) that restrict the pharmacy's ability to stock products or provide services that meet the therapeutic needs of the community.

1.2.4.2 The pharmacy shall have a Professional Services Area that is distinguishable from the general trading area.

1.2.4.3 The pharmacy shall provide a healthy and safe working environment for pharmacy staff that meets jurisdictional requirements.

Note:

Pharmacies are required to comply with the mandatory requirements from Safe Work Australia's Compliance Code: Workplace Amenities and Work Environment relevant to their State or Territory.

1.2.4.4 The pharmacy shall maintain a security access register which includes details of who has access to the keys and/or codes to the pharmacy and safes at any given time.

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Reflective Questions

- Is the pharmacy premises maintained appropriately to ensure the safe and professional conduct of the business?
- Do we have a designated Professional Services Area that is distinguishable from the general trading area?
- Are we providing a healthy and safe working environment for staff that meets Safe Work Australia's Compliance Code: Workplace Amenities and Work Environment requirements?
- Do we maintain a security access register detailing who has access to the keys and/or codes to the pharmacy and safes at any given time?

Key Tasks

- Undertake routine inspections of the pharmacy premises to identify any maintenance concerns and perform scheduled maintenance and repairs to maintain the safety of the pharmacy environment.
- If the property is leased, ensure the terms of the lease support the operation of the pharmacy and do not restrict the pharmacy's ability to stock products or provide services.
- Establish a designated Professional Services Area that is distinctly separate from the general trading area, facilitates private and health-focused conversations, and can be supervised by a pharmacist.
- Equip the Professional Services Area with adequate furnishings and resources to facilitate confidential consultations and display scheduled medicines and health-related products.
- Adhere to Safe Work Australia's Compliance Code: Workplace Amenities and Work Environment requirements to ensure the pharmacy is a safe and healthy place for staff to work in.
- Provide regular training for staff on health and safety protocols to ensure a safe working environment.
- Establish and maintain a Security Access Register of personnel with keys and/or security code information for the pharmacy and safes.
- Implement secure access controls, as appropriate to the size of the business, to protect sensitive areas within the pharmacy.

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Monitoring and Evidence Examples

- Maintenance schedules and/or logs showing regular inspections and repairs of the pharmacy premises.
- Lease agreements that align with pharmacy operations, ensuring no restrictions on stocking products or providing services.
- Records of routine property maintenance to ensure safety and professionalism.
- Demonstrated evidence of a designated Professional Services Area separate from the general trading area that can be supervised by a pharmacist.
- Evidence of adherence to Safe Work Australia's Compliance Code: Workplace Amenities and Work Environment requirements, which could include (but is not limited to) workplace health and safety policies for the pharmacy, rosters and timesheets outlining meal breaks for staff, and facilities for staff such as amenities, drinking water and personal storage.
- Training records, indicating staff participation, in sessions related to health and safety protocols.
- The pharmacy's Security Access Register detailing personnel with access to keys and/or codes to the pharmacy and safes.
- Records of secure access controls implemented within the pharmacy.

Resources and Information

Safe Work Australia

- Comprehensive guidelines on workplace amenities, health, and safety protocols.

WorkSafe Victoria

- Compliance Code: Workplace Facilities and the Working Environment



4.5 Safety and Security

Intention

The pharmacy is a safe and secure environment, with systems in place to identify and manage potentially dangerous or hazardous situations and emergencies.

Outcome

I feel safe when I am in my community pharmacy.

Standard Subclause

The pharmacy shall have a system to identify and manage potentially dangerous or hazardous situations and emergencies to ensure the safety and security of pharmacy staff, consumers and the business.

1.2.5.1 The pharmacy shall maintain a system to ensure the effective safety of consumers, pharmacy staff and the pharmacy by:

a. Identifying and documenting potential risks and hazards, both physical and psychological.

b. Implementing measures to control, prevent or minimise potential risks and hazards.

c. Following a process to respond to any hazardous situations or emergencies if they occur.

d. Recording and reporting (if appropriate) the details of any hazardous situations or emergencies.

e. Monitoring, reviewing and evaluating preventative measures to ensure they remain effective.

Note:

Risks and hazards may pose a threat to a person's physical or psychological wellbeing.

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Standard Subclause

Employers have a legal responsibility and a duty of care to ensure the health and safety of workers, and to ensure that others are not put at risk from work carried out by a business. While physical injuries are easily recognised, psychological, or psychosocial hazards are often not as obvious. Common psychosocial hazards include role overload, conflict or poor workplace relations, poor support from managers and colleagues, bullying, harassment, racism and exposure to traumatic events.

Note:

Pharmacies are required to comply with requirements of Safe Work Australia, including Safety Data Sheet requirements for hazardous substances as required.

The pharmacy shall have processes to educate pharmacy staff of potentially dangerous situations and emergencies, facilitate reporting of hazardous situations and ensure pharmacy staff have appropriate qualifications to assist in the event of a medical emergency.

2.4.2.1 All members of the pharmacy staff shall be educated on:

a. Potential risks and hazards in the pharmacy and their role in controlling, preventing and minimising them.

b. How to report risks, dangerous and hazardous situations that may arise.

2.4.2.2 The pharmacy shall ensure there are members of the pharmacy staff with appropriate and current qualifications in First Aid and CPR as required for the services the pharmacy provides.

a. The pharmacy should have access to a defibrillator and ensure pharmacy staff are able to use the defibrillator in the case of an emergency.

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Reflective Questions

- What measures have we implemented to control, prevent, or minimise identified actual and potential risks and hazards?
- Do we have processes for recording and reporting details of any hazardous situations or emergencies, as appropriate?
- How often do we monitor, review, and evaluate our preventative measures to ensure strategies to minimise risks and hazards remain effective?
- Are all members of our pharmacy staff educated on potential risks and hazards and their roles in controlling them?
- What strategies do we have in place to ensure pharmacy staff have current qualifications in First Aid and CPR?
- Is there access to a defibrillator in the pharmacy, and are staff trained to use it in an emergency?



Key Tasks

- Systematically identify and document potential risks and hazards. This includes both physical and psychological hazards that could affect staff, consumers, or the business environment (e.g. fire, floods, armed robbery, manual handling risks).
- Once risks are identified, implement appropriate measures to control, prevent, or minimise these risks. This may involve physical adjustments to the workplace, changes in procedures, or training for staff.
- Establish, maintain and follow emergency response procedures to effectively respond to hazardous situations or emergencies. This includes having clear protocols for action, ensuring staff know these procedures, and regularly practicing emergency drills.
- Facilitate access for pharmacy staff to support services (e.g. trauma counselling, employee assistance programs), as needed.
- Ensure up to date First Aid and CPR training for adequate members of staff as required to maintain emergency preparedness and support applicable pharmacy services (e.g. vaccination services).
- Provide access to emergency equipment such as anaphylaxis kits (as required), fire extinguishers and a defibrillator.

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Monitoring and Evidence Examples

- Documented risk assessments outlining potential risks and hazards and measures to control, prevent, or minimise them.
- The pharmacy's emergency response protocols outlining steps to be taken in various emergency scenarios appropriate for the pharmacy.
- Logs of emergency drills conducted, including participant lists, dates, and outcomes.
- Records in the pharmacy's incident register of hazardous situations or emergencies that have occurred in the pharmacy, including descriptions, dates, actions taken, outcomes, and to whom they were reported (as applicable).
- Safety audit reports assessing potential risks and hazards and the effectiveness of preventative measures.
- Training records for pharmacy staff related to potential risks and hazards, their role in preventing them, and how to report risks and hazards that arise.
- Documented evidence or staff meeting minutes where Workplace Health & Safety is discussed.
- Demonstrated evidence that pharmacy staff have access to trauma counselling and/or an employee assistance program if they require support.
- Records of current First Aid and CPR qualifications for pharmacy staff members and evidence that qualified staff are available when applicable pharmacy services are being delivered (e.g. vaccination services).
- Demonstrated evidence that pharmacy staff have access to emergency equipment such as an anaphylaxis kit (as required), fire extinguishers and a defibrillator.
- Records of training for First Aid and CPR qualified staff members related to the use of anaphylaxis kits (as required) and the defibrillator.
- Records of fire extinguisher and defibrillator maintenance and checks to ensure functionality.



Resources and Information

Telephone Support Service for Pharmacists

- Pharmacists' Support Service.

National Institute for Health and Care Excellence

- Violence and aggression: short-term management in mental health, health and community settings.

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4.6 Information Technology (IT)

Intention

The IT systems in place in the pharmacy support the provision of services and accurate and secure recording of healthcare data.

Outcome

I am confident that my personal information is used appropriately for my benefit, and data is protected and stored securely.

Standard Subclause

The pharmacy shall have systems and processes to ensure the appropriate selection, management and accessibility of IT equipment, systems, and data.

1.2.6.1 Owners and/or managers of the pharmacy shall select IT systems that:

a. Are reputable and fit for purpose.
b. Incorporate safeguards against cyber intrusion.
c. Safeguard the integrity and confidentiality of data.
d. Comply with legislative requirements regarding data storage.
e. Protect the availability of the system.

1.2.6.2 Owners and managers of the pharmacy shall maintain relevant IT equipment, systems and communication infrastructure that promote effective and efficient workflows and operations and ensure data security is protected on an ongoing basis.

Note:

IT systems may be located onsite in the pharmacy or off-premises. Regardless of their location, the pharmacy shares the responsibility of ensuring data security with any vendors it engages.

1.2.6.3 The pharmacy shall proactively prepare a plan to be followed in the event of a data breach or cyber security incident, including a transparent approach to communicating with all stakeholders.

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Standard Subclause

The pharmacy shall have systems and processes to ensure the appropriate access to IT equipment, systems and data and to prevent breaches in data privacy and loss of system access.

2.4.3.1 The pharmacy shall promote a culture of cyber resilience by:

a. Developing a cyber strategy, proactively overseen by the pharmacy owners and managers.

b. Setting clear roles and responsibilities in relation to cyber resilience.

c. Training pharmacy staff to ensure they have the knowledge and skills to utilise the IT systems in place and that they understand the importance of data privacy and cybersecurity.

d. Promoting strong cyber security practices.

2.4.3.2 The pharmacy shall implement and maintain processes to ensure:

a. Access to IT systems and data is restricted to authorised personnel.

b. There is no unauthorised or unsupervised access to confidential data held or accessed by the pharmacy.

2.4.3.3 The pharmacy shall implement, maintain and follow a process to recover and restore IT systems and data in the event of infrastructure loss, or loss of system access.

Reflective Questions

- How well does our IT system support the operational needs of the pharmacy?
- Are our IT systems and equipment appropriate for the size and complexity of our pharmacy?
- Have we evaluated the reputation and reliability of our current IT vendors?
- What measures are currently in place to ensure data integrity and confidentiality?
- Are our data privacy and cybersecurity protocols aligned with relevant legal and regulatory requirements?
- Do we restrict access to IT systems and data solely to authorised personnel effectively?
- What steps have we taken to foster a culture of cyber resilience among staff?
- Do we provide staff with regular training on cybersecurity and data privacy best practices?
- How prepared are we to respond to a cyber security incident or data breach?
- Do we have a clear and actionable plan to recover IT systems and data in the event of system failure?
- Have we tested our recovery processes to ensure they work effectively under real-world conditions?
- What mechanisms are in place to regularly review and update our IT policies and practices?
- Are our workflows optimised to leverage the capabilities of our IT systems?
- How do we ensure ongoing compliance with evolving standards and regulations?

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Key Tasks

- Maintain IT equipment and systems to promote efficient workflows while safeguarding system access, data integrity and confidentiality. This should include selecting IT systems that:
  - **Are reputable.**
  - **Are tailored to the pharmacy's operational needs.**
  - **Incorporate safeguards against cyber intrusion.**
  - **Comply with legislative requirements regarding data storage.**
- Implement robust cybersecurity measures and data security protocols to protect sensitive data.
- Prepare a plan for responding to cybersecurity incidents, including clear communication strategies for affected stakeholders and compliance with regulatory reporting requirements.
- Determine what level of IT system access each pharmacy staff member requires and restrict access accordingly to prevent unauthorised access to confidential data.
- Establish and maintain a procedure and recovery plan to restore IT systems in the event of a data breach or infrastructure loss.
- Provide ongoing training to pharmacy staff, ensuring they are equipped with the necessary knowledge to use IT systems effectively, uphold cybersecurity practices and understand the importance of data privacy.
- Regularly review IT systems and processes, which may include the reputation and reliability of IT vendors, to ensure IT systems remain effective, efficient, and compliant with current standards.

Monitoring and Evidence Examples

- Demonstrated evidence that the pharmacy's IT systems have safeguards against cyber intrusion.
- Demonstrated evidence of regular maintenance of the pharmacy's IT equipment and systems.
- The pharmacy's documented procedures and recovery plans, which outline:
  - **Steps to restore IT systems and data after a system failure or cyber-attack.**
  - **Adherence to regulatory reporting requirements for cyber incidents.**
  - **Communication strategies to all stakeholders.**
- The pharmacy's IT policies and procedures, including cyber-security and data security protocols to protect patient health information.
- IT system access control records which identify restricted access to IT systems, detailing authorised personnel and access levels.
- Records of staff training and education related to the use of the pharmacy's IT systems, the pharmacy's IT policies and procedures, cybersecurity and data security practices, recovery plans, and staff roles and responsibilities.
- Records and outcomes of simulated recovery exercises conducted to verify the effectiveness of IT recovery processes.
- Incident reports which reflect past responses to cybersecurity incidents (if any), including corrective actions taken.
- Records of system performance reviews and/or reports evaluating IT systems' reliability, efficiency, and alignment with operational requirements.

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Resources and Information

Australian Government Australian Signals Directorate

- Australian Cyber Security Centre

Australian Government Office of the Australian Information Commissioner

- Australian Privacy Principles
- Guide to Health Privacy
- Health Service Providers
- Healthcare Identifiers

Australian Digital Health Agency

- Conformant clinical software products
- Electronic prescribing for dispensers
- Electronic Prescribing Conformance Register



4.7 Advertising of Pharmacy Services and Therapeutic Goods

Intention

Advertising materials for medicines and pharmacy services are factual, ethical and comply with relevant codes, guidelines and legislation.

Outcome

I trust the health care information provided to me by my community pharmacy is accurate, ethical and evidence based.

Standard Subclause

The pharmacy shall have systems to ensure that advertising of pharmacy services is factual, ethical and complies with relevant codes, guidelines and legislation.

2.1.3.1 The pharmacy shall ensure any clinical or advertising claim is supported by evidence.

2.1.3.2 The pharmacy shall ensure all advertising and promotional material is accurate, balanced, does not mislead and is not capable of leading the consumer to unrealistic expectations.

Standard Subclause

The pharmacy shall ensure that advertising or promotion of therapeutic goods is factual, ethical and complies with relevant codes, guidelines and legislation.

2.2.5.1 The pharmacy shall ensure any clinical claims:

a. Are consistent with the body of evidence applicable to the products.
b. Are clearly communicated and easily understood by the intended audience.
c. Identify and cite scientific or clinical research appropriately (as applicable).

2.2.5.2 The pharmacy shall ensure that all advertising material includes mandatory statements and is:

a. Accurate.
b. Balanced.
c. Not likely to mislead consumers.
d. Substantiated prior to dissemination.
e. Aligned with information held in the Australian Register of Therapeutic Goods.
f. Not promoting inappropriate or excessive use.
g. Not capable of leading consumers to unrealistic expectations.
h. Not inconsistent with public health campaigns.

2.2.5.3 The pharmacy shall ensure that the dissemination of price information relating to therapeutic goods complies with relevant codes and guidelines.

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Reflective Questions

- How does the pharmacy ensure that all advertising and promotional materials are accurate, balanced, and do not mislead patients and consumers?
- What processes are in place to verify that clinical claims in pharmacy advertising are supported by robust evidence and appropriately cited?
- How does the pharmacy ensure that advertising information is being conveyed clearly, is balanced, and easily understood by the target audience?
- Are regular evaluations of advertising practices undertaken to align with the latest codes, guidelines, and legislation?
- Do we consider consumer feedback regarding our advertising to ensure trust and transparency?
- Do we have a mechanism for reviewing advertising materials to ensure they are free from unrealistic claims and expectations?
- How do we educate pharmacy staff about the ethical and legal responsibilities related to advertising pharmacy services?
- Does the way we disseminate price information comply with relevant codes and guidelines while ensuring clarity for consumers?

Key Tasks

- Develop and implement systems to ensure advertising of therapeutic goods and pharmacy services is:
  - **Supported by evidence.**
  - **Accurate and balanced.**
  - **Clear and easy to understand.**
  - **Not misleading or likely to lead consumers to unrealistic expectations.**
  - **Not promoting inappropriate or excessive use of therapeutic products.**
  - **Aligned with public health campaigns.**
  - **Compliant with relevant Codes, legislation and guidelines.**
- Establish a mechanism for reviewing all advertising and promotional materials to check they are accurate, balanced, and do not mislead consumers. This may include verifying that any clinical or advertising claims are supported by robust, current, evidence-based research and cited accordingly.
- Regularly evaluate advertising practices to align with updates to codes, guidelines, and legislation.
- Incorporate consumer feedback into evaluations to enhance trust and transparency in advertising.
- Educate pharmacy staff about ethical and legal responsibilities related to advertising.
- Regularly review price information to ensure it is transparent and compliant with relevant codes, guidelines and legislation.

Note

While the development and review of advertising materials is often completed centrally (e.g. by Banner Group or Head Office staff, or pharmaceutical companies), community pharmacies still retain responsibility for the advertising materials displayed in their pharmacies. Pharmacies need to be satisfied that all advertising materials are compliant, and patient safety is not compromised.

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Monitoring and Evidence Examples

- The pharmacy's documented advertising policy outlining processes, and responsibilities, to ensure advertising materials are ethical, accurate, balanced, and compliant with regulations.
- Demonstrated evidence that promotional materials in the pharmacy are transparent and compliant.
- Records of consumer feedback related to advertising and promotion including whether it has been integrated into advertising strategies to enhance transparency.
- Records of staff training related to the ethical and legal responsibilities of advertising therapeutic goods and pharmacy services.
- Compliance checklists used to review promotional materials.
- Demonstrated evidence of audits of promotional material verifying adherence to the ethical and legal requirements for advertising therapeutic goods and services.
- Demonstrated evidence that price lists and other pricing information is clear, transparent, promotes easy comparison for consumers and is compliant.

Resources and Information

Australian Government Therapeutic Goods Administration

- Therapeutic Goods Act 1989
- Therapeutic Goods Regulations 1990
- Therapeutic Goods Advertising Code
- The Poisons Standard (the SUSMP)

Pharmaceutical Society of Australia

- Code of Ethics for Pharmacists

Australian Competition and Consumer Commission

- Pricing

Australian Health Practitioner Regulation Agency

- Advertising Hub

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4.8 Stock Management

Intention

The community pharmacy has robust stock management systems to ensure the availability, appropriate storage and disposal of medicines and therapeutic goods.

Outcome

I am confident that the medicines I need are readily available and have been stored correctly to protect the integrity of the products.

Standard Subclause

The pharmacy shall have systems for ordering, storing, and disposing of therapeutic goods that align with jurisdictional requirements, maintain the integrity of the medicines and support the safe provision of medicines.

2.2.1.1 The pharmacy shall maintain and follow a system for ordering, receiving, unpacking and pricing stock.

2.2.1.2 The pharmacy shall not stock, sell or promote:

a. Tobacco, or tobacco/smoking-related products or implements (this does not refer to therapeutic products used to aid smoking cessation).

b. Alcoholic beverages, home brew or alcohol distilling kits, or products that promote the use of alcohol.

c. Products with no evidence-based therapeutic benefit which do not enhance the practice of pharmacy.

2.2.1.3 The pharmacy shall ensure that no product stocked or supplied by the pharmacy, or used for the delivery of services, is out of date or will become out of date during a normal course of use.

2.2.1.4 The pharmacy shall:

a. Ensure no product supplied, or consumables used for the delivery of services, are subject to a recall.

b. Maintain and follow a system for initiating, managing and reporting products subject to recall.

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Standard Subclause

2.2.1.5 The pharmacy shall comply with professional requirements, manufacturer's instructions, and legislative and jurisdictional requirements for:

a. The identification of high-risk medicines within the pharmacy.

b. Safe and secure storage of medicines, including scheduled medicines, controlled medicines, cytotoxic medicines, medicines identified as high risk and other stock items.

c. The stability of medicines while stored in the pharmacy.

d. Storage of temperature-sensitive medicines and cold chain management.

e. Storage and safe disposal of unused, unwanted or expired medicines including controlled drugs, high-risk medicines, cytotoxic medicines, needles and other sharps.

2.2.1.6 The pharmacy shall ensure it aligns with the National Vaccine Storage Guidelines 'Strive for 5' and other professional guidelines to prevent a cold chain breach of temperature-sensitive medicines.

Reflective Questions

- How does the pharmacy review current stock management practices to ensure they align with legislation, jurisdictional requirements and professional standards?
- What mechanisms are in place to ensure we are not promoting the use of products with no evidence-based therapeutic benefit?
- What measures are in place to ensure the safe storage, handling, and disposal of high-risk medicines, temperature-sensitive items, and controlled drugs?
- How effectively does the pharmacy monitor the expiry dates of products to prevent the distribution of outdated medicines?
- What processes are implemented to identify and manage recalled products, and are these systems robust enough to prevent errors?
- How does the pharmacy ensure adherence to the National Vaccine Storage Guidelines 'Strive for 5' for temperature-sensitive medicines?
- How do we ensure that all medicines (including temperature sensitive medicines) are stored and handled according to manufacturers' directions?
- How does the pharmacy manage and report risks associated with the storage of medicines?
- What processes are in place to ensure we dispose of medicines in a safe and compliant manner?

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Key Tasks

- Establish and maintain a structured ordering and inventory management system for ordering, receiving, unpacking and storing therapeutic goods.
- Establish and embed processes to identify high-risk medicines (e.g. controlled medicines, cytotoxic medicines) and handle and store them in line with their schedule, professional requirements, manufacturer's instructions and legislative and jurisdictional requirements.
- Establish, maintain and follow a system for cold chain management to maintain the integrity of temperature-sensitive medicines that adheres to the National Vaccine Storage Guidelines 'Strive for 5'.
- Implement a system to ensure products with limited evidence of therapeutic benefit are provided with appropriate information and advice to allow consumers to make an informed choice about purchasing.
- Establish, maintain and follow expiry management systems and processes to ensure that no products in stock are outdated or nearing expiry during their normal course of use.
- Implement robust systems for initiating, managing, and reporting recalled products to avoid errors.
- Establish, maintain and follow a system for the safe and compliant return and disposal of unused, unwanted, or expired medicines, including controlled substances and cytotoxic products.
- Train and educate pharmacy staff on the legal and professional requirements regarding stock management and the safe and appropriate storage of medicines.

Monitoring and Evidence Examples

- The pharmacy's documented policies and procedures for inventory management, including ordering, receiving, unpacking and storing therapeutic goods.
- The pharmacy's procedures for identifying, handling and storing high-risk medicines.
- Demonstrated evidence of therapeutic goods storage areas to confirm secure and appropriate organisation of scheduled, high-risk and controlled medicines.
- Demonstrated evidence that tobacco and alcohol-containing products are not stocked by the pharmacy.
- The pharmacy's procedure for cold chain management, including a protocol to be followed in the event of a cold chain breach.
- Dispensary refrigerator temperature monitoring records to verify compliance with the National Vaccine Storage Guidelines 'Strive for 5'.
- Documented approval from State Health Department to hold National Immunisation Program (NIP) stock (if applicable).
- The pharmacy's protocol for ensuring products with limited evidence of therapeutic benefit are provided safely and transparently.
- Logs or records tracking the expiry dates of products to ensure outdated medicines are not distributed.
- Evidence of the pharmacy's process for managing recalled products.
- The pharmacy's procedure for the return and disposal of unused, unwanted, or expired medicines (including controlled substances).
- Records of staff training related to the legal and professional requirements regarding stock management and the safe and appropriate storage of medicines.

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Resources and Information

Australian Government Therapeutic Goods Administration

- The Poisons Standard (the SUSMP)

Australian Government Department of Health, Disability and Ageing

- National Vaccine Storage Guidelines ‘Strive for 5’
- National Return and Disposal of Unwanted Medicines Program

Pharmaceutical Society of Australia

- State and Territory Legislation links



4.9 Human Resources

Intention

The community pharmacy values its staff and has structured recruitment, training, development, and performance management procedures in place to foster a competent and motivated workforce.

Outcome

I receive professional care from the staff within my community pharmacy.

Standard Subclause

The pharmacy shall have processes to employ, manage, train and develop pharmacy staff, as well as to manage voluntary and involuntary departures of members of the pharmacy staff.

2.4.1.1 The pharmacy shall have a process for the recruitment of members of the pharmacy staff that includes:

a. A recruitment procedure.
b. Developing position descriptions for all positions and using these as the basis for any selection process.
c. Maintaining records of interviews conducted with short-listed applicants.
d. Providing successful applicants with a written contract of employment defining the terms and conditions of engagement, prior to commencing employment.
e. Collecting and storing a signed copy of the contract of employment for all successful applicants.
f. Advising unsuccessful applicants that they have not been selected for employment.

2.4.1.2 The pharmacy shall ensure there is a formal agreement in place to support pharmacy students undertaking work integrated placements in the pharmacy.

Note:

Universities and Vocational Education and Training (VET) providers should provide a formal agreement template during the process of establishing a student's placement.

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Standard Subclause

Pharmacies should also confirm pharmacy-appropriate professional indemnity arrangements are in place for students undertaking placements in the pharmacy.

2.4.1.3 The pharmacy shall have processes for the oversight, supervision and management of members of the pharmacy staff which include:

a. An induction process that is completed within three months of a new member of the pharmacy staff commencing employment.

b. Assessing that new members of the pharmacy staff can apply all policies and procedures applicable to their role as outlined in the induction process.

c. Supervising and managing the performance of new members of the pharmacy staff during their qualifying/probationary period.

d. A performance review process to monitor, inform and manage pharmacy staff performance.

e. Maintaining a pharmacy staff roster that provides sufficient capability to support the safe and appropriate provision of medicines and pharmacy services and is compliant with relevant industrial instruments (for example the Pharmacy Industry Award).

f. Seeking feedback from pharmacy staff on issues affecting them and/or their work environment.

g. A formal workplace grievance process that is utilised when required.

h. Developing and maintaining a personnel file for each member of the pharmacy staff, ensuring confidentiality and privacy of all personnel information.

2.4.1.4 The pharmacy shall implement a process to oversee providers contracted by a third party to deliver a health service on behalf of the pharmacy.

2.4.1.5 The pharmacy shall systematically plan and deliver ongoing training for pharmacy staff by:

a. Maintaining a training and development plan for each member of the pharmacy staff which outlines their training needs relevant to the positions they hold.

b. Providing resources and opportunities for members of the pharmacy staff to meet their identified training needs.

c. Providing members of the pharmacy staff with continuous access to training on products, services and procedures relevant to the pharmacy's business model and the positions they hold.

d. Maintaining a training record for each member of the pharmacy staff that details all training offered and undertaken.

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Standard Subclause

2.4.1.6 The pharmacy shall have a process to manage dismissals and resignations of members of the pharmacy staff that includes:

a. Recording the reasons for dismissals or resignations.

b. Offering all departing members of the pharmacy staff an exit interview, recording that the offer was made and keeping records of the interview in their personnel file.

c. Offering departing members of the pharmacy staff appropriate final documents including a statement of service and a copy of their training and development record.

d. Retaining records of dismissals and resignations for the required period in accordance with legislation.

2.4.1.7 Where possible, the pharmacy should utilise feedback provided by departing members of the pharmacy staff to make improvements to the operation of the pharmacy.

Reflective Questions

- What steps has the pharmacy implemented to ensure that recruitment practices and processes are transparent, fair and aligned with workplace relations legislation?
- How do we ensure that all employment contracts are comprehensive and securely stored to protect both the pharmacy and the staff?
- Are the induction and training processes thorough enough to prepare new staff members for their roles effectively? How do we assess their understanding of policies and procedures?
- What mechanisms do we use to monitor staff performance and provide feedback that enhances their professional growth?
- How accessible and effective is the staff grievance process? Does it foster a positive work environment?
- How do we oversee services provided by third-party service providers to ensure they meet expected quality and compliance standards?
- How do we ensure staff have continuous access to relevant training and development opportunities? Are training records maintained accurately for future reference?
- How do we utilise feedback from departing staff to improve our pharmacy operations and work culture?
- How do we consistently ensure that all personnel files and records comply with confidentiality and privacy legislation?

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Key Tasks

- Develop and follow a systematic process for staff recruitment that is fair, equitable and consistently applied.
- Develop position descriptions for each role within the pharmacy and include role-appropriate responsibilities for safety and quality. The position descriptions, and selection criteria, should be utilised when recruiting new staff members.
- Develop questions for recruitment interviews that cover selection criteria, role responsibilities and behavioral questions.
- Provide written employment contracts, and relevant Fair Work Information Sheets, to successful applicants prior to commencing employment, which clearly define terms and conditions of employment, and comply with relevant industrial relations (IR) laws.
- Retain, and securely store, records of recruitment and employment contracts as per legislation.
- If accepting pharmacy students to undertake work integrated placements in the pharmacy, obtain and keep a record of placement information from the relevant University and ensure there is a process to follow if an incident occurs affecting the student.
- Implement and follow a comprehensive induction process for new staff, including agency and locum members within the first three months of employment, this is to include:
  - **Orientation to safety, quality and clinical governance policies and processes;**
  - **Evaluation of the new staff member's ability to apply policies and procedures relevant to their roles during the induction period, and**
  - **Supervision to manage performance during the probationary period.**
- Conduct regular performance reviews for all staff members.
- Maintain a staff roster to ensure adequate capability for safe and effective pharmacy services.
- Solicit regular feedback from staff regarding work-related issues and the work environment.
- Establish a formal workplace grievance procedure to address workplace concerns. Ensure staff are aware of the process.
- Keep and update confidential personnel files for all staff.
- Develop and implement structured procedures for the oversight and management of any third-party providers engaged to deliver healthcare services. Roles, responsibilities, objectives of the engagement and insurance requirements should be clearly defined.
- Develop and document training and development plans tailored to each staff member's roles and needs.
- Facilitate ongoing access to training on products, services, and procedures relevant to staff positions.
- Establish and follow a process when a staff member leaves the pharmacy due to resignation or dismissal. The process should include:
  - **Documenting reasons for dismissals or resignations.**
  - **Offering exit interviews to all departing staff and recording relevant observations.**
  - **Providing appropriate final documents, including statements of service and training records.**
  - **Retaining dismissal and resignation records as required by legislation.**
  - **Utilising feedback from departing staff to improve pharmacy operations.**

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Monitoring and Evidence Examples

- The pharmacy's procedure or checklist that guides the recruitment process for pharmacy staff.
- Pharmacy staff position descriptions, which include safety and quality responsibilities.
- Recruitment interview pro-forma which includes behavioural questions.
- Recruitment records which include job descriptions, interview notes, and a signed employment contract for each pharmacy staff member.
- Records of integrated student placement arrangements and processes from relevant Universities (if applicable).
- The pharmacy's induction procedure and checklists, and documented evidence of completed induction checklists.
- Records of regular performance appraisals and reviews that align with the pharmacy's performance review process.
- Up-to-date staff rosters that demonstrate sufficient and capable staffing for the activities offered by the pharmacy.
- Evidence of regular staff feedback initiatives (e.g. surveys, staff meeting minutes, diary or communications book, electronic messaging platforms, notice board) addressing workplace concerns.
- The pharmacy's workplace grievance procedure and records of workplace grievances, resolutions, and relevant correspondence (where a grievance has been reported).
- Demonstrated evidence of how staff files (e.g. contracts, performance reviews etc) are kept secure and confidential.
- A documented contract management procedure for assessing and managing third-party providers.
- Documented training and development plans tailored to the staff roles, alongside records of completion for training activities.
- The pharmacy's procedure for resignations and dismissals.
- Retained copies of resignation and dismissal records maintained according to legislative requirements.
- Completed exit interview forms and feedback reports from departing staff members.
- Documented evidence that an exit interview was offered and declined.

Resources and Information

Fair Work Ombudsman

- Fair Work Act 2009
- Fair Work Commission
- Pharmacy Award [MA000012]

Government of Western Australia

- WA Retail Pharmacists' Award
- Shop and Warehouse (Wholesale and Retail Establishments) State Award

Australian Government, Business

- Employment Contract Tool

Other resources

- Australian Small Business and Family Enterprise Ombudsman

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QUALITY & SAFETY PHARMACY PROGRAM

Appendix

Appendix 1: Glossary

Terms	Definition
Adverse drug reaction	A response to a medicine that is noxious and unintended and occurs at doses normally used or tested in humans for the prophylaxis, diagnosis or therapy of disease, or for the modification of physiological function. (AS 85000:2024)
Adverse event	An incident that results, or could have resulted, in harm to a patient or consumer. A near miss is a type of adverse event. (ACSQHC's Primary and Community Healthcare Standards)
Aseptic technique	A set of practices aimed at minimising contamination, used particularly to protect patients from infection during procedures. (AS 85000:2024)
Business continuity plan	A plan to help a pharmacy respond to a crisis event, minimise interruption, recover and resume normal operations as quickly as possible. A crisis event may be a fire, flood, loss of key staff, pandemic etc.
Clinical governance	The accountability of pharmacy owners in ensuring that there is a strong culture and appropriate systems and processes in place to support the safety, quality and continuous improvement of all activities and services provided in or from a community pharmacy. It involves relevant and appropriate relationships and responsibilities to be established between pharmacy owners, their staff, patients, consumers and other stakeholders to support good clinical outcomes for patients. It ensures that community pharmacies and their patients can be confident that systems are in place to deliver safe, quality and patient-centred health care. (AS 85000:2024)
Community pharmacy	Also known as a retail pharmacy, a service that provides pharmaceutical products, services, and health advice in the community setting. (AS 85000:2024)
Community Pharmacy Vending Machine (CPVM)	A secure mechanised storage unit that enables automated, self-service selection and payment transactions to supply a non-scheduled medicines or other unrestricted pharmacy product.

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Complex compounding	Complex compounding requires or involves special competencies, equipment, processes and/or facilities. Examples include sterile preparations, preparations containing ingredients that pose an occupational health and safety hazard (such as cytotoxics or some hormones) and micro-dose single-unit dosage forms containing less than 25 mg (or up to 25 per cent by weight or volume) of active ingredient. Refer to the section Compounding in the current edition of the Australian Pharmaceutical Formulary and Handbook for further examples and information. (From Pharmacy Board of Australia Guideline on Compounding October 2024)
Conflict of interest	In pharmacy, a conflict of interest is a circumstance where a pharmacist's professional judgment in patient care, or their professional duties, is at risk of being unduly influenced by a secondary interest, such as financial gain, rather than solely by the patient's best interests. These secondary interests can be actual, potential, or perceived and are not inherently unethical, but they must be openly disclosed and effectively managed to maintain public trust and the integrity of the profession.
Consent	Permission granted voluntarily by a consumer or individual who has been adequately informed (e.g. of options, risks, benefits) and has the capacity to understand, provide and communicate their permission. (PSA's Competency Standards for Pharmacists in Australia 2016).
Consumer	A person who has used, or may potentially use, pharmacy services, or is a carer or family member of a patient using pharmacy services. (AS 85000:2024)
Controlled medicines	Schedule 8 medicines; medicines with strict legislative requirements for storage and supply.
Cytotoxic medicines	Medicines which have a deleterious effect upon cells and may be mutagenic, teratogenic or carcinogenic. These medicines are predominantly used to treat cancer or autoimmune disorders and require special handling.
Employment documentation	Employment documents can include position descriptions, duty statements, employment contracts, performance review documentation and notification of scope of clinical practice.
First aid qualification	The first aid qualification is to be the 'Senior First Aid' certificate and should be current as per the accredited training provider's guidelines.

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Hazardous materials	An item or agent, which has the potential to cause harm, whether by itself or with other factors.
Health promotion	A process/activity where the pharmacy actively engages consumers and the community to promote health and wellbeing at a group or population level.
Healthcare providers	An individual who practices a profession relating to the provision of health care. Healthcare providers may be required to maintain profession-specific registration with a national board under the National Registration and Accreditation Scheme or be self-regulated. A healthcare provider may also be referred to as a health practitioner, clinician, or profession-specific description. (AS 85000:2024)
Healthcare services	A separately constituted organisation that is responsible for implementing clinical governance, administration and financial management of a service unit or service units providing health care to patients. It can be in any location or setting, including pharmacies, clinics, outpatient facilities, hospitals, patients' homes, community and primary healthcare settings, practices and clinicians' rooms. (ACSQHC's Primary and Community Healthcare Standards)
High-risk medicines	Medicines that have an increased risk of causing significant patient harm or death if they are misused or used in error. High-risk medicines may vary between healthcare settings, depending on the types of medicines used and patients treated. Errors with these medicines are not necessarily more common than with other medicines. Because they have a low margin of safety, the consequences of errors with high-risk medicines can be more devastating. At a minimum, the following classes of high-risk medicines should be considered: • Medicines with a narrow therapeutic index. • Cytotoxic medicines. • Medicines that present a high risk when other system errors occur, such as administration via the wrong route. • Schedule 8 medicines. • Medicines with a greater potential for addiction or abuse. (AS 85000:2024)
Internal audit	Self-reflection, clinical audit, financial audit, review of records.

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Medical devices	Therapeutic goods that can be used to diagnose, prevent, treat and monitor medical conditions. They include surgical equipment, syringes, gloves, nebulisers, vaporisers, pacemakers, baby incubators and implants. The Therapeutic Goods Administration (TGA) regulates the supply of medical devices in Australia. (AS 85000:2024)
Open disclosure	An open discussion with a patient and carer about an incident that resulted in harm to the patient while receiving health care. The criteria of open disclosure are an expression of regret and a factual explanation of what happened, the potential consequences and the steps taken to manage the event and prevent recurrence. (AS 85000:2024)
Patient	A person who is receiving health care from a healthcare service such as a pharmacy. (AS 85000:2024)
Patient healthcare record	A record of a patient's medical history, treatment notes, observations, correspondence, investigations, test results, prescription records and medication charts for an episode of care. (AS 85000:2024)
Pharmacy Collection Unit (PCU)	A secure mechanised unit that a community pharmacy can use, only after a patient consult with the pharmacist or delegated staff member, to allow collection of medicines and other pharmacy goods that have been prepared and paid in advance of collection by identified individuals.
Pharmacy staff	All people working in a pharmacy including owners, managers, pharmacists, intern pharmacists, pharmacy technicians, dispensary assistants, pharmacy assistants and any other people employed or contracted by the pharmacy such as locum pharmacists, third party providers, other healthcare professionals, students or volunteer workers.
Poisons	An ingredient, compound, material or preparation which, or the use of which, may cause death, illness or injury and includes any ingredient, compound, material or preparation referred to in a schedule to the current Poisons Standard. (Poisons Standard (SUSMP))

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Policy documents	Policy documents can include policies, procedures, protocols and guidelines. A policy document may exist for a single action, several actions and/or part of a standard sub-clause. The number of policies and detail in each policy will depend on the pharmacy's size, complexity and type of services.
Policy	A course or principle of action adopted or proposed by an organisation or individual. (AS 85000:2024)
Prescribing	An iterative process involving the steps of information gathering, clinical decision-making, communication and evaluation which results in the initiation, continuation or cessation of a medicine. (AS 85000:2024)
Procedure	A sequential set of steps which describes a process for completing a task or set of tasks. (AS 85000:2024)
Protocols	An established set of rules used to complete tasks or a set of tasks. (AS 85000:2024)
Reputable pharmaceutical supplier	A supplier of raw materials that holds relevant licenses with the Therapeutic Goods Administration (TGA), is accredited and able to provide certificates of analysis (CoA), batch records, and material safety data sheets for every raw material supplied.
Risk management	The design and implementation of a program to identify and avoid or minimise risks to patients, employees, volunteers, visitors and the organisation. (AS 85000:2024)
Risk management plan	A formal, proactive document that outlines a pharmacy's systematic approach to identifying, assessing, prioritising, and mitigating potential risks that could impact the pharmacy. It details the strategies, tools, and processes for managing risks, including defining roles, responsibilities, and how risks will be monitored and controlled.

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Safety Data Sheets (SDS)	A document that provides comprehensive information about the identity, hazards, health effects, safe handling, storage, emergency procedures, and disposal considerations of hazardous chemicals. It serves as a crucial tool for workers and employers to understand and manage the risks associated with these substances, ensuring workplace and environmental safety.
Scope of practice (of a pharmacist)	A time-sensitive, dynamic aspect of practice which indicates those professional activities that a pharmacist is educated, competent and authorised to perform and for which they are accountable. (AS 85000:2024)
Standard precautions	Work practices that provide a first-line approach to infection prevention and control and are used for the care and treatment of all patients. Standard precautions include: hand hygiene, use of personal protective equipment (masks, gloves, gowns, protective eyewear) to prevent blood or bodily fluid exposure, routine environmental cleaning aligned to risk, safe use and disposal of sharps, reprocessing of reusable equipment and devices, respiratory hygiene and cough etiquette (including physical distancing), aseptic technique, linen and waste management. (ACSQHC's Primary and Community Healthcare Standards)
Sterile compounding	A highly controlled, specialised process of preparing medications in a contaminant-free environment to prevent infection when administered directly into the body. It's crucial for medications like injections, IVs, and eye drops that bypass the body's natural defences, requiring the use of sterile equipment, cleanroom facilities, and rigorous aseptic techniques to ensure patient safety and product purity.
Third-party providers	An independent individual or organisation that delivers services, products, or support to the pharmacy but is not an employee.
Transmission-based precautions	Extra work practices used in situations when standard precautions alone may not be enough to prevent transmission of infection. Transmission based precautions are used in conjunction with standard precautions and include droplet, contact and airborne precautions or a combination of these precautions based on the route of transmission of infection. (ACSQHC's Primary and Community Healthcare Standards)
Vaccination and administration services	The process of administering a vaccine or other therapeutic preparation into the body. Vaccines are the materials used for immunisation, including extracts of killed or live weakened strains of viruses or bacteria. Immunisation is the process of inducing immunity to an infectious agent by administration of a vaccine. (adapted from PSA's Practice Guidelines for pharmacists providing immunisation services)

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Links & Resources

Appendix 2: Useful Organisations and Resources

Organisation/ Resource	Outline
Australian Small Business and Family Enterprise Ombudsman (ASBFEO)	The ASBFEO helps small businesses and family enterprises to resolve disputes involving other businesses or Commonwealth Government agencies and avoid costly formal legal avenues. This includes activities to advocate for small businesses and family enterprises to improve policies affecting them.
Australasian College of Pharmacy (ACP)	ACP develops and delivers tailored education (such as continuing professional development), training and research programs to pharmacists and the pharmacy sector. ACP also sets quality education and credentialling standards and offers professional support and networking opportunities to its members.
Australian Government Australian Signals Directorate (ASD)	ASD provides foreign signals intelligence, cyber security, and offensive cyber operations to defend Australia from global threats and to advance national interests. The agency collects and analyses foreign communications and data, provides advice and assistance to protect Australian government, businesses, and individuals from cyber threats, and conducts offensive cyber capabilities to disrupt offshore threats.
Australian Government Office of the Australian Information Commissioner (OAIC)	The OAIC is an independent statutory agency with three primary functions: • To act as the national regulator for privacy and freedom of information, providing a complaint resolution mechanism for individuals. • To advise the government on information policy, particularly in the digital and AI age. • To ensure the proper handling of personal information under the Privacy Act 1988. The OAIC investigates breaches, handles complaints, reviews agency decisions, and promotes standards for information management to build public trust in government information.
Australian Health Practitioner Regulation Agency (AHPRA)	AHPRA works with National Boards (such as the Pharmacy Board of Australia) in regulating registered health practitioners in Australia. AHPRA manages the National Registration and Accreditation Scheme, maintaining a public register of practitioners and responding to complaints.

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Australian Pharmacy Council (APC)	APC is an independent national accreditation authority for pharmacy education and training. APC accredits pharmacy programs and continuing professional development accrediting organisations and assesses pharmacists' skill and knowledge through qualification checks and exams.
Pharmacy Board of Australia (PBA)	The national regulator of pharmacists in Australia whose primary role is to protect the public by ensuring only qualified, competent, and safe pharmacists practice in Australia. The Board also develops regulatory standards, codes, guidelines, updates and other resources for the pharmacy professional and manages complaints and disciplinary matters.
Australian Commission on Safety and Quality in Health Care (the Commission)	Leads national efforts to improve the safety and quality of healthcare in Australia by developing national standards, clinical guidelines and overseeing accreditation programs. The Commission works in partnership with governments, clinicians and the community to achieve a safer and higher-quality health system for all Australians.
Australian Digital Health Agency (ADHA)	Responsible for the operation and delivery of high quality, trusted, reliable, and secure national digital health infrastructure and health support systems including My Health Record.
Capacity Australia	Capacity Australia is a not-for-profit charity that promotes autonomy of decision-making. Promoting support to ensure that where a person lacks capacity to make their own decisions, the necessary safeguards are in place to prevent abuse, neglect and exploitation. The group has dedicated experts from the medical, legal, ageing and disability fields with experience in capacity, guardianship, disability, mental health and old age psychiatry, and are available to provide training and assistance to all States and Territories and internationally.
Fair Work Ombudsman	The Fair Work Ombudsman promotes harmonious, productive and compliant workplace relations by: • providing education, assistance, advice and guidance to employers, employees and organisations • promoting and monitoring compliance with workplace laws • inquiring into and investigating breaches of the Fair Work Act and taking appropriate enforcement action.

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Healthdirect Australia	A government-funded, free, virtual health service that provides trusted health information, advice, and referral services to Australians via a telephone helpline, website and app.
National Aboriginal Community Controlled Health Organisation (NACCHO)	NACCHO serves as the national leadership and peak body for Aboriginal Community Controlled Health Organisations (ACCHOs) in Australia, advocating for policy and budget improvements, facilitating community-driven health solutions, providing comprehensive primary healthcare, and fostering partnerships to improve the health and wellbeing of Aboriginal and Torres Strait Islander peoples.
National Safety and Quality Primary and Community Healthcare Standards	These person-centred standards describe the processes and structures that primary and community health care services need to adopt in order to deliver safe and high-quality health care to Australian communities.
National Vaccine Storage Guidelines 'Strive for 5'	These Guidelines provide information and advice for cold chain management of vaccines to ensure consumers receive effective and potent vaccines.
Network of Alcohol and other Drugs Agencies (NADA)	NADA is the peak organisation for non-government alcohol and other drug services in New South Wales.
Pharmaceutical Defence Limited (PDL)	A member-based organisation that supports pharmacist members with Professional Indemnity (PI) insurance and risk management advice.
Pharmacy Industry Award [MA000012] (Pharmacy Award)	The Pharmacy Award outlines the minimum employment standards for staff in the community pharmacy sector including pharmacists, pharmacy students and pharmacy assistants.
Pharmaceutical Society of Australia (PSA)	The PSA is a national member-based training organisation representing Australian pharmacists and provides resources and support for their professional development and career progression.

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Pharmacy Guild of Australia (the Guild)	The Guild advocates for and supports community pharmacy owners across Australia by promoting the vital role of pharmacies in the healthcare system, negotiating funding agreements, and providing professional development and business support for its members.
Advanced Pharmacy Australia (AdPha)	Formerly the Society of Hospital Pharmacists of Australia (SHPA), AdPha supports hospital pharmacists to deliver medicines-related services and improve patient care and health outcomes through professional development, clinical standards and advocacy.
Therapeutic Goods Act 1989	A national system for the control and regulation of medicines, medical devices, and other biological products to ensure their safety, quality, and efficacy. The Act provides a legal framework for the import, export, manufacturing, and supply of therapeutic goods and requires them to be included in the Australian Register of Therapeutic Goods (ARTG).
Therapeutic Goods Administration (TGA)	The TGA regulates the supply, import, export, manufacturing and advertising of medicines, medical devices, and biologicals to ensure they are safe, effective and of high quality for consumers. They evaluate and assess new products before they can be sold, monitor safety and compliance, and take action against illegal activities like counterfeiting. The TGA also maintains the Australian Register of Therapeutic Goods (ARTG) and licenses manufacturers to ensure their products meet strict quality and safety standards.
Therapeutic Goods Advertising Code	The Therapeutic Goods Advertising Code sets out standards for advertising therapeutic goods (like medicines and medical devices) to the public in Australia, ensuring these advertisements are ethical, legal, and don't mislead or deceive consumers. The Code carries legal penalties for non-compliance under the Therapeutic Goods Act 1989.
WorkSafe Australia	The national policy-making body for work health and safety (WHS) and workers' compensation in Australia. WorkSafe Australia develops and evaluates national strategies to reduce work-related death, injury and illness.

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